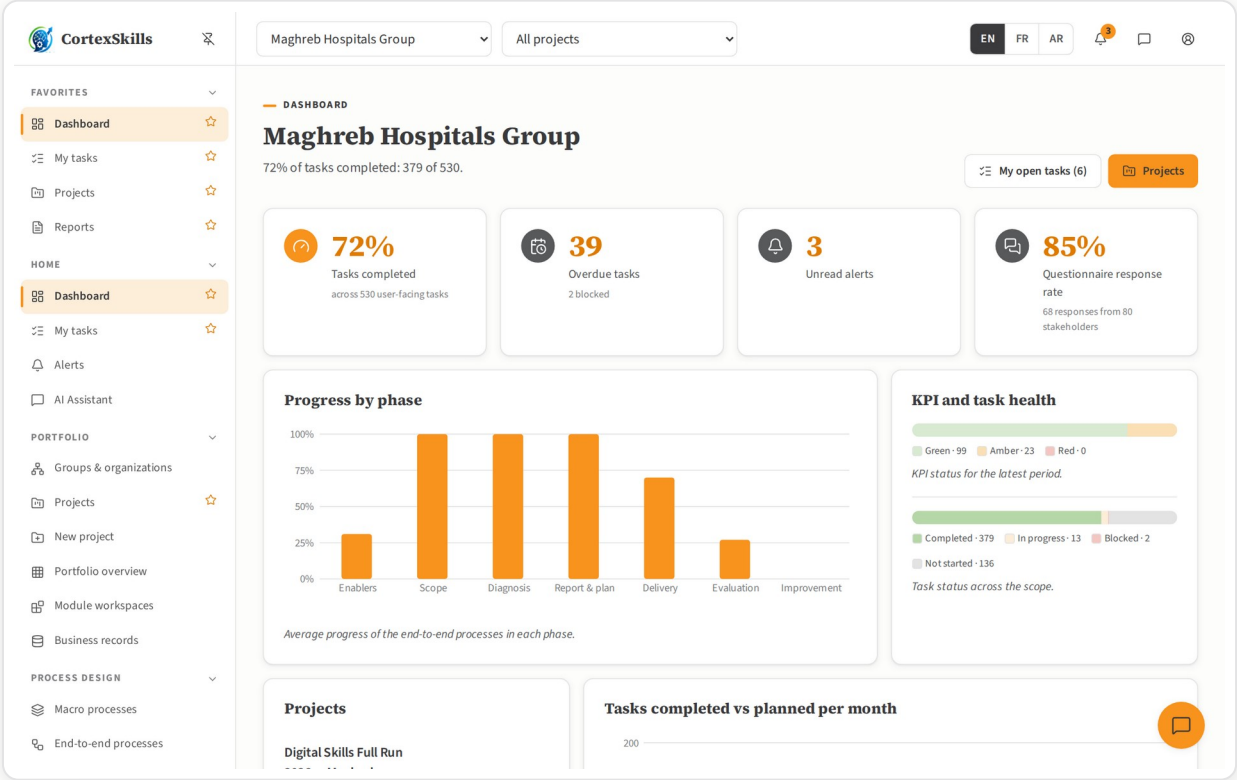


CortexSkills for Healthcare

Training engineering for hospitals and clinics — from accreditation requirements to certified, evaluated staff



What this presentation covers



01

The healthcare challenge

Accreditation, patient safety and new skills



02

The solution

Lifecycle, gates, healthcare processes



03

Case study: hospital group

A full AI skills run, phase by phase



04

Case study: a clinic

The same method on an SME track



05

The health ecosystem

Life sciences and pharma



06

Trust and next steps

Security, deployment, packs, plan

One hospital group, one AI skills run, measured end to end

Maghreb Hospitals Group · 4,700 employees · Casablanca · figures from the seeded demonstration run



65%

of 265 tasks completed in the AI skills run



3 of 6

phase gates signed off with a “Go” decision



34

questionnaire responses, 91% average completeness



1,260,000

training budget in the locked 2026 plan

What the run shows



Diagnosis flagged 11 of 21 function × decision-level cells as Red; Clinical Care & Nursing is Red at strategic and operational levels.



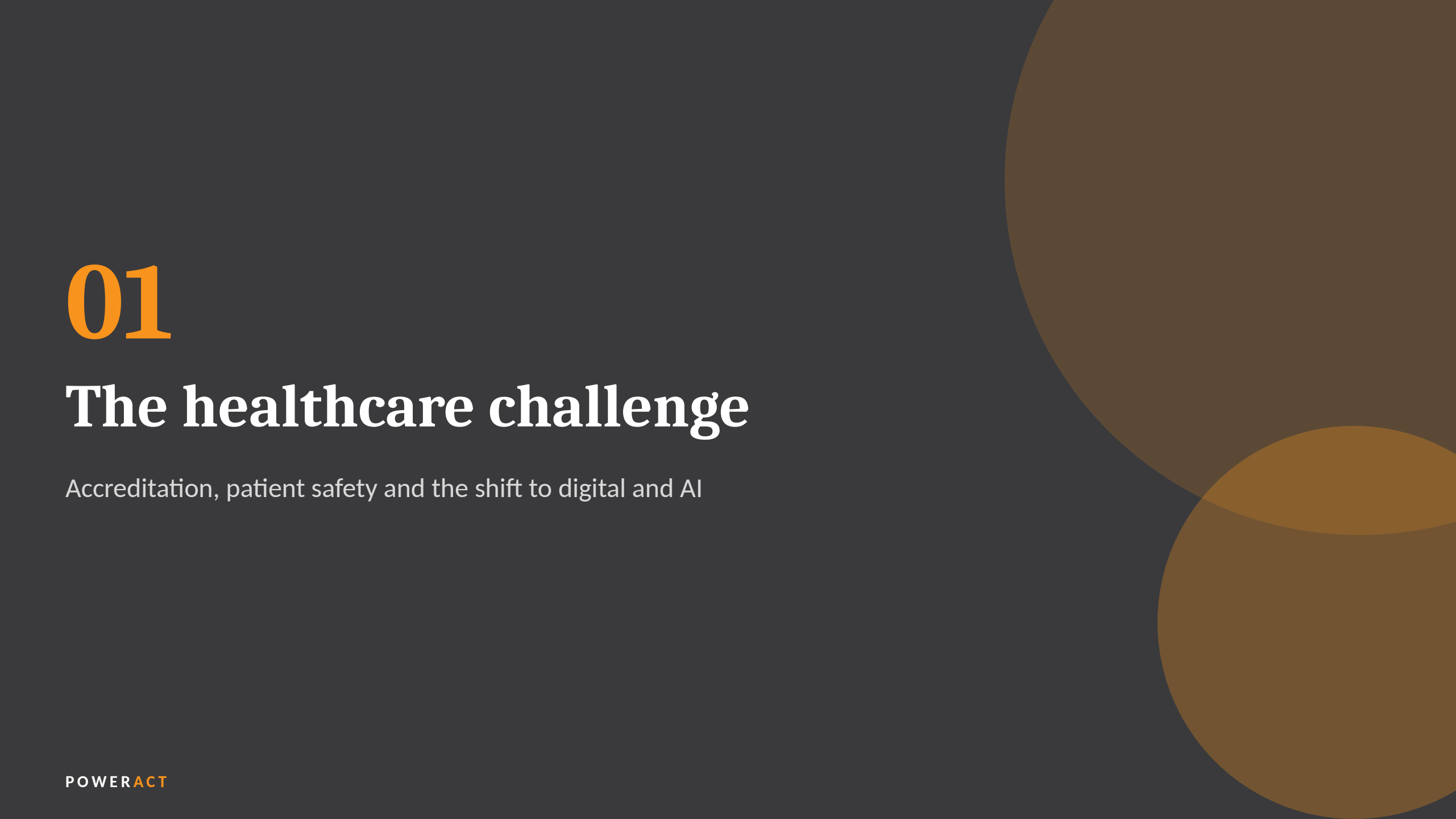
Six AI themes were prioritized by impact and urgency; “AI triage and clinical decision support” ranks first (impact 5, urgency 4).



Kirkpatrick evaluations range from 74% to 93%; one ISO 7101 awareness certificate has expired and one is expiring.



Every one of the 265 RACSI activities has exactly one Accountable; 381 audit entries trace the changes.



01

The healthcare challenge

Accreditation, patient safety and the shift to digital and AI

Patient safety depends on proven competence

The accreditation bodies ask for evidence that each role is trained, assessed and current.



ISO 7101

Healthcare quality management: competence determined per role, training records kept, effectiveness evaluated.



JCI Standards

Accreditation surveys check staff qualification and education, with evidence at the level of each practitioner.



ISO 15189

Medical laboratories: competence of laboratory personnel, assessed and recorded.

SECTOR DRIVER

Patient-safety accreditation burden

Every accreditation cycle requires competence evidence for regulated roles.

SECTOR RISK

Adverse clinical event linked to a training gap

Reviewed at each gate through the healthcare readiness checklist.

What Learning & Development teams in hospitals must solve



Clinical competence

Map every Clinical Care & Nursing position to its obligations and keep certificates valid.



Audit-ready evidence

Attendance, results and manager observations kept per step, retrievable for a survey.



Digital and AI adoption

Electronic records, telemedicine, AI triage and imaging change daily practice.



Multi-site, multilingual

Several sites and three working languages — English, French and Arabic.

Four healthcare verticals, each with its own standards

Each vertical brings its macro processes, end-to-end processes and readiness checklist.

HCPR

Healthcare Providers

Standards

ISO 7101
JCI Standards
ISO 15189

Core function

Clinical Care & Nursing

Demo: Maghreb Hospitals Group

HLS

Healthcare & Life Sciences

Standards

ISO 13485
EU MDR
ISO 15189

Core function

Clinical & Laboratory
Operations

Demo: Maghreb Life Sciences

PHAR

Pharma

Standards

EU GMP
GDP
ICH Q9

Core function

Pharmaceutical Quality &
Distribution

Demo: Maghreb Pharma Distribution

PHPR

Pharmaceutical & Process

Standards

EU GMP
ICH Q10
FDA 21 CFR Part 11

Core function

GMP Manufacturing

Demo: Pharma Maghreb Process

Priority skills themes, digital and AI

Seeded for the Healthcare Providers vertical; each organization adjusts them during diagnosis.



Digital skills

- 1 Electronic health records (EHR)
- 2 Telemedicine and remote consultation
- 3 Digital patient scheduling and billing



AI skills

- 1 AI triage and clinical decision support
- 2 AI medical imaging assistance
- 3 AI bed and staff capacity planning

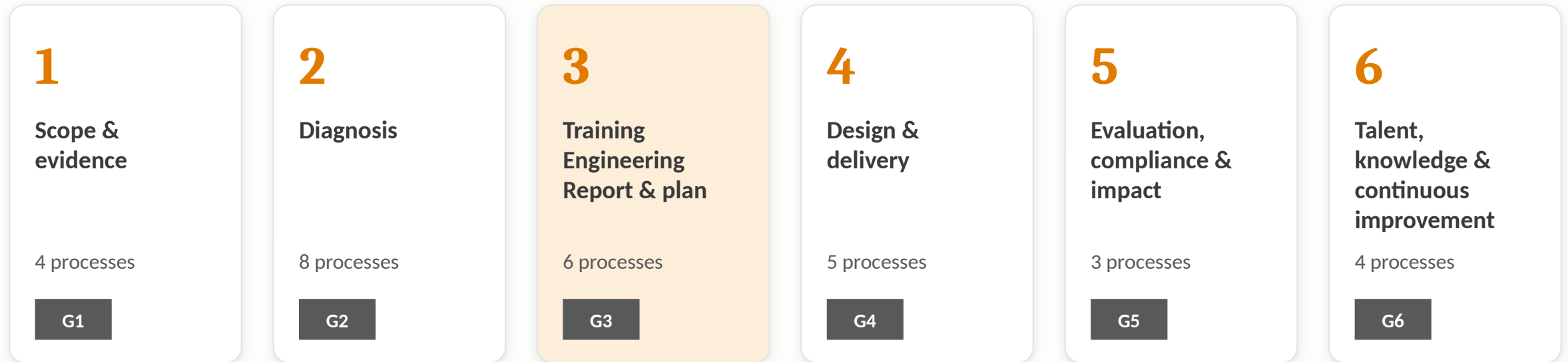
02

The solution

One lifecycle, seven phases, gates, and healthcare-specific processes

One lifecycle, seven phases

Each phase groups end-to-end processes; a gate closes each main phase.



Enablers run across all phases — data, AI and integration • change and adoption • quality, risk and improvement

Six gates, each with a checklist and named approvers

A phase closes only when the mandatory items are met and the decision is recorded.

Gate	Purpose	Approvers
G1 — Scope & evidence approved	Confirm the approved Scope of Work and a consolidated questionnaire dataset.	HR Director, Consultant PM
G2 — Diagnostic validated	Validate diagnostic findings, gaps, demands, RAG and SWOT.	Head of L&D, HR Director
G3 — Report & roadmap approved	Approve the Training Engineering Report, prioritized plan and budget.	HR Director, Steering Committee
G4 — Delivery readiness	Confirm designs, content, trainers and logistics before delivery.	Head of L&D
G5 — Evaluation & impact validated	Validate effectiveness, ROI and compliance status.	Head of L&D, Finance Manager
G6 — Closure & continuous improvement	Close the cycle with REX and the post-engagement roadmap.	HR Director, Partner

Three healthcare macro processes on top of the core model

Added automatically when the organization's sector is Healthcare Providers.



HCPR-MP-01

Regulatory & Safety Training Compliance

Keep every regulated role in Healthcare Providers trained and certified against ISO 7101.

Owner: Compliance Officer



HCPR-MP-02

Critical Skills & Certification Framework

Define the critical Clinical Care & Nursing skills and their certification paths.

Owner: Head of L&D



HCPR-MP-03

Digital & AI Skills Transformation

Plan the digital and AI skills that Healthcare Providers operations will need.

Owner: L&D Strategist

Two healthcare end-to-end processes

Each has a trigger, an end state, the macro processes it crosses and its user-facing tasks.

HCPR-E2E-01

Compliance-to-Certification Readiness

Goal	Keep Healthcare Providers regulated roles certified against ISO 7101.
Trigger	New ISO 7101 requirement or certification expiry.
End state	Sector compliance extract issued with every critical role certified.
Crosses	MP-01 · MP-02 · MP-14 · HCPR-MP-01
Tasks	4 user-facing tasks

HCPR-E2E-02

Critical-Skills-to-Transformation Plan

Goal	Turn critical-skill gaps and digital and AI use cases into a transformation plan.
Trigger	Annual plan cycle or technology change in Healthcare Providers.
End state	Transformation learning paths approved.
Crosses	MP-02 · MP-03 · MP-05 · HCPR-MP-02
Tasks	4 user-facing tasks

A readiness checklist for Healthcare Providers

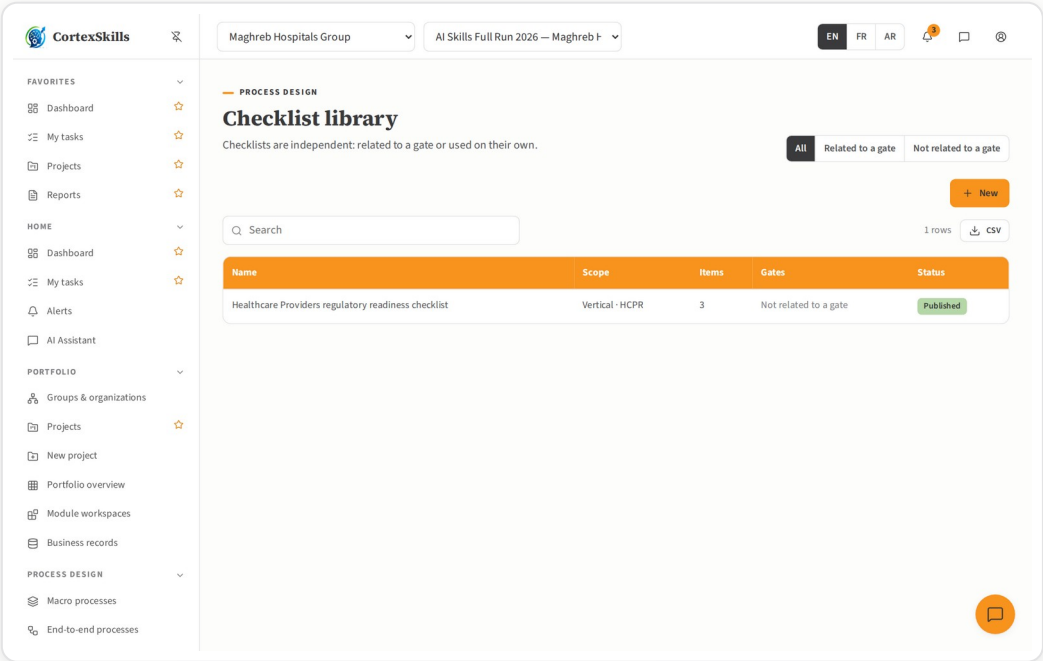
Published once for the vertical; used at the gates and in the compliance extract.

Checklist item	Mandatory	Evidence
ISO 7101 training requirements mapped to positions	Yes	Required
Critical Clinical Care & Nursing roles certified	Yes	Required
Sector risk reviewed: Adverse clinical event linked to a training gap	Yes	—



Readiness support, not certification

The application organizes evidence and checks readiness against ISO 7101, JCI and ISO 15189. The certification decision stays with the accreditation body.



29 roles, one Accountable per activity

RACSI: Responsible, Accountable, Consulted, Supported, Informed — allocated for every activity of the run.

CortexSkills

FAVORITES

Dashboard

My tasks

Projects

Reports

HOME

Dashboard

My tasks

Alerts

AI Assistant

PORTFOLIO

Groups & organizations

Projects

New project

Portfolio overview

Module workspaces

Business records

PROCESS DESIGN

Macro processes

End-to-end processes

Maghreb Hospitals Group

AI Skills Full Run 2026 — Maghreb

ENFRAR

3

GOVERNANCE & RISK

RACSI

Responsible, Accountable, Consulted, Supported, Informed — one Accountable per activity.

☐ R — Responsible

☐ A — Accountable

☐ C — Consulted

☐ S — Supported

☐ I — Informed

Q Search

265 rows

CSV

Activity	Name	Step	R	A	C	S	I	
UFT-01-01	Record strategic objectives	MP-01.1	Head of L&D	HR Director	CEO	L&D Strategist	CHRO	+
UFT-01-02	Link objectives to functions and sites	MP-01.2	L&D Strategist	Head of L&D	Function Heads	Analyst	CHRO	+
UFT-01-03	Frame skills priorities	MP-02.1	L&D Analyst	Head of L&D	HR Business Partner	Analyst	CHRO	+
UFT-01-04	Pre-scope training needs	MP-03.1	L&D Analyst	Head of L&D	Managers	Analyst	CHRO	+
UFT-01-05	Define budget envelope	MP-01.3	Finance Manager	CFO	Head of L&D	Analyst	CHRO	+
UFT-01-06	Plan change and adoption	MP-19.1	Change Manager	Head of L&D	Business Leaders	Analyst	CHRO	+
UFT-01-07	Compile plan approval pack	MP-01.4	Head of L&D	HR Director	Finance Manager	Admin	Steering Committee	+
UFT-01-08	Approve and lock	MP-01.5	HR Director	CHRO	Head of L&D	Admin	All Stakeholders	+

Examples of roles

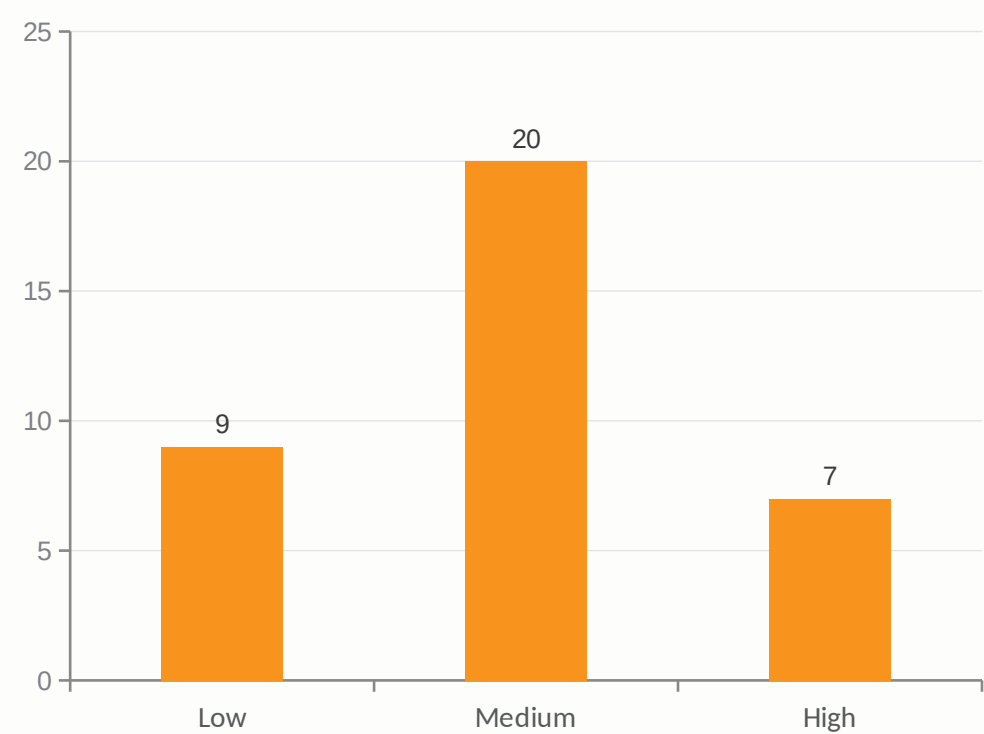
- HR Director / Steering Committee Chair
- Head of L&D
- L&D Analyst
- L&D Strategist
- L&D Business Partner
- L&D Planner
- Instructional Designer
- Training Administrator

265

activities in the hospital run, each with exactly one Accountable

36 AI use cases, each reviewed by a person

AI drafts; a named role accepts, edits or rejects. Every suggestion is logged with its outcome.



Number of catalog AI use cases by risk level

Use case	Risk	Human checkpoint
Training Engineering Report drafting	High	L&D Strategist edits every section; Quality Manager reviews before approval.
Diagnostic findings drafting	High	Consultant PM validates findings with the client sponsor.
Theme prioritization scoring	High	L&D Strategist confirms the ranking; Steering Committee validates the plan.
Skill gap forecasting (6-18 months)	High	Head of L&D accepts or rejects each prediction before any action.
Content translation draft (FR/AR)	Medium	Subject Matter Expert validates terminology before publication.

03

Case study: a hospital group

Maghreb Hospitals Group — 4,700 employees, Casablanca. Demonstration data, fictional organization.

Maghreb Hospitals Group

Member of Maghreb Health & Life Group • Healthcare Providers • large company

**4,700**
employees

**7**
functions in the organization structure

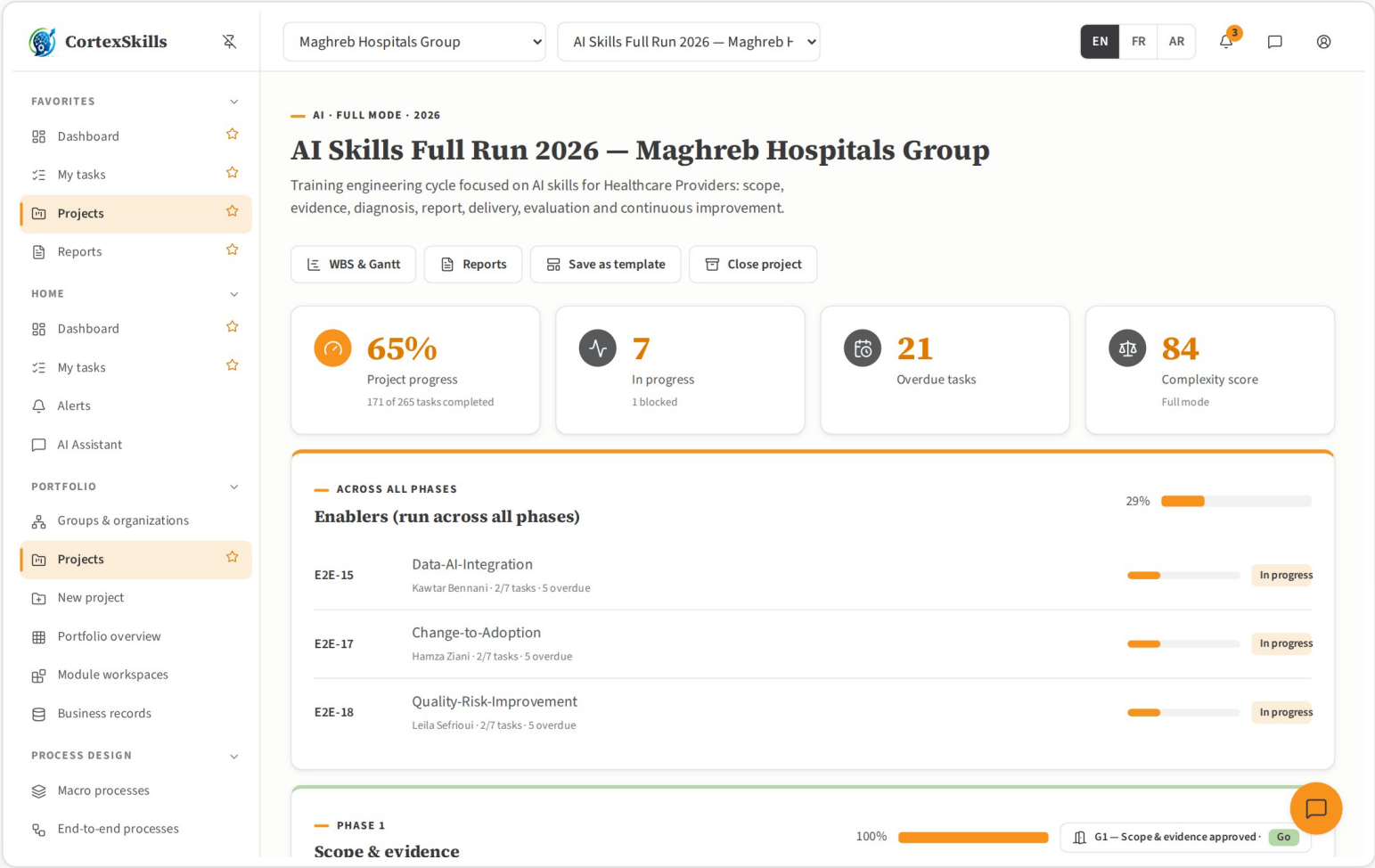
**29**
named users, one per role

**2**
full runs in 2026: Digital and AI

Setting	Value
Functions	Operations, Quality & HSE, Finance, Human Resources, Sales & Marketing, Information Systems, Clinical Care & Nursing
Solution	BND-05 Full Suite (PK-01 to PK-08)
Add-ons	Multi-Site / Multi-Entity Deployment; Questionnaire Library & Multi-Channel Distribution
Data protection frameworks	GDPR, Law 09-08 (CNDP, Morocco), ISO 9001
Deployment • seats	SaaS • 600 seats
Justification on governed changes	Required

The AI skills run on one screen

AI Skills Full Run 2026 — Maghreb Hospitals Group



65% of 265 tasks completed; 7 in progress, 1 blocked.



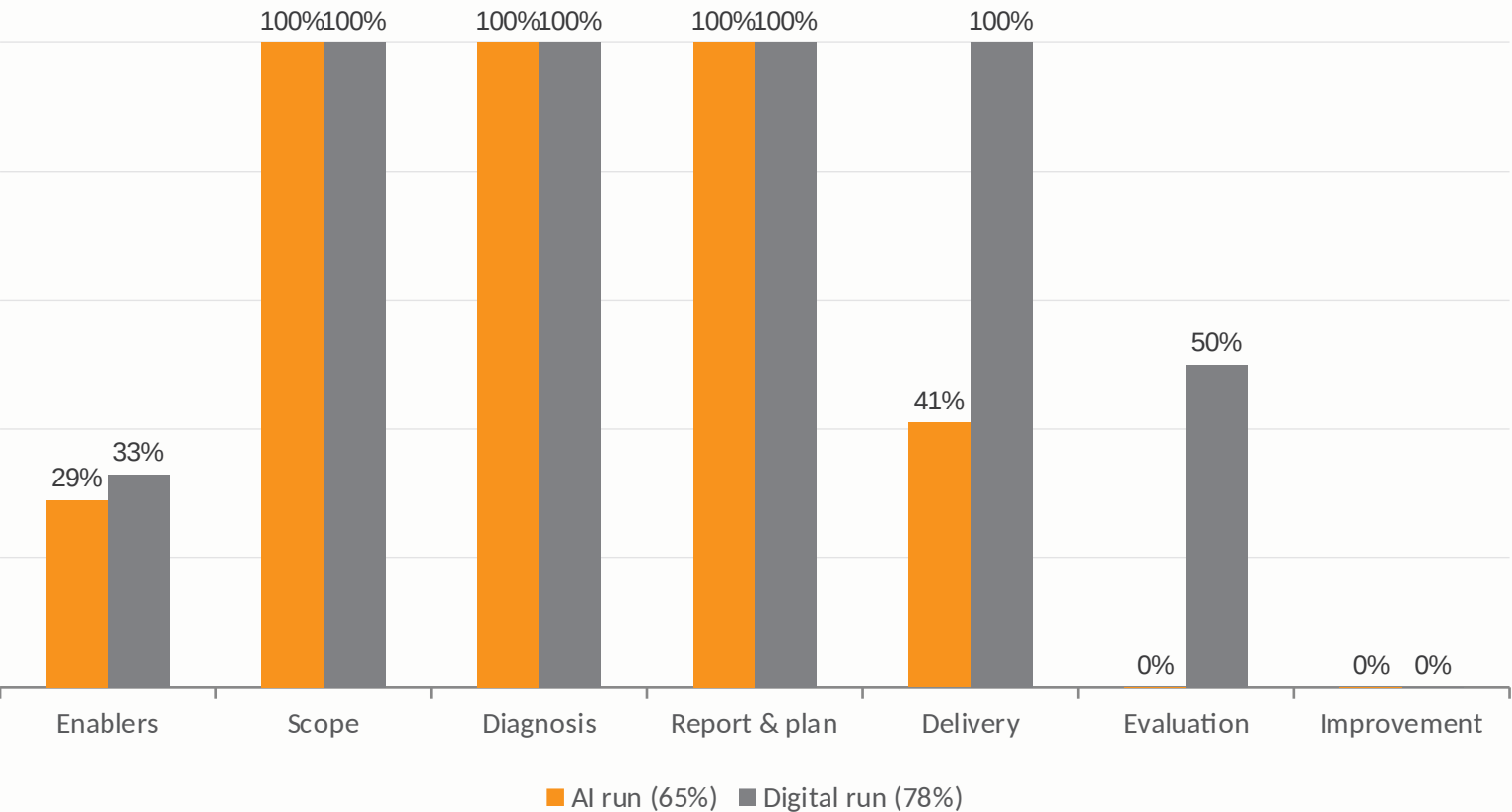
21 tasks overdue — each raises an alert with an escalation chain.



Complexity score 84: Full mode, all seven phases and six gates.

Two runs: the Digital run leads on delivery and evaluation

Share of tasks completed per phase, same organization, same template.



Tasks completed / tasks planned in each phase (%)



208 / 265

tasks completed in the Digital run



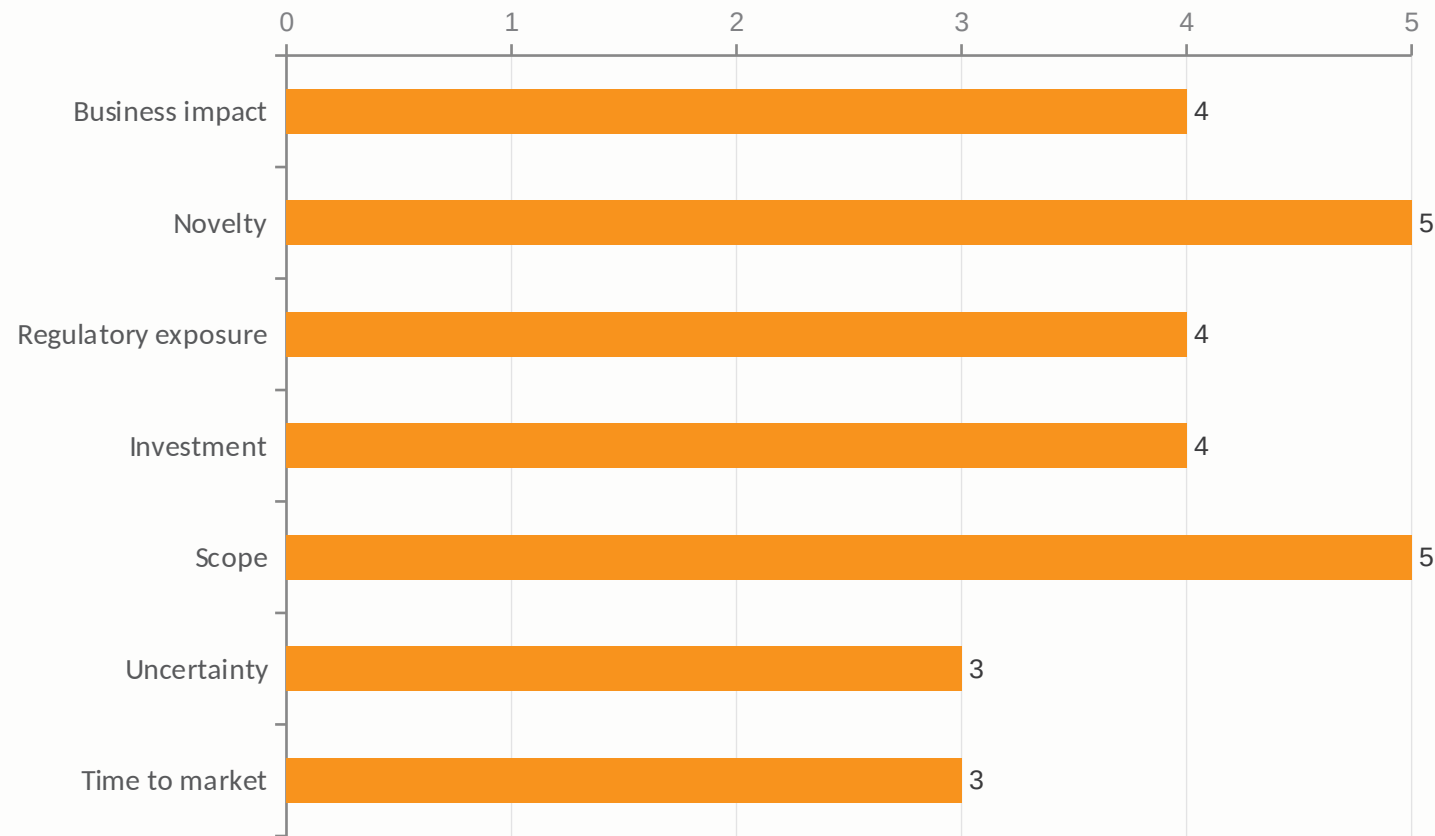
171 / 265

tasks completed in the AI run

Scope, diagnosis and the report are complete in both runs; the gap opens in delivery and evaluation.

Complexity score 84 out of 100: Full mode

Seven weighted criteria rated 1-5 when the project is created; the score recommends the mode.



Rating of each complexity criterion (1 = low, 5 = high)

84

Full mode: score of 70 or more

Below 70, the score recommends SME mode and one of three tracks (T1, T2, T3). Choosing a different track requires a written justification.

A scope approved before any data is collected

Approved on 15 Jul 2026 • focus AI • decision levels MS / MO / OP

**17**

scope criteria: function × decision level

**40**

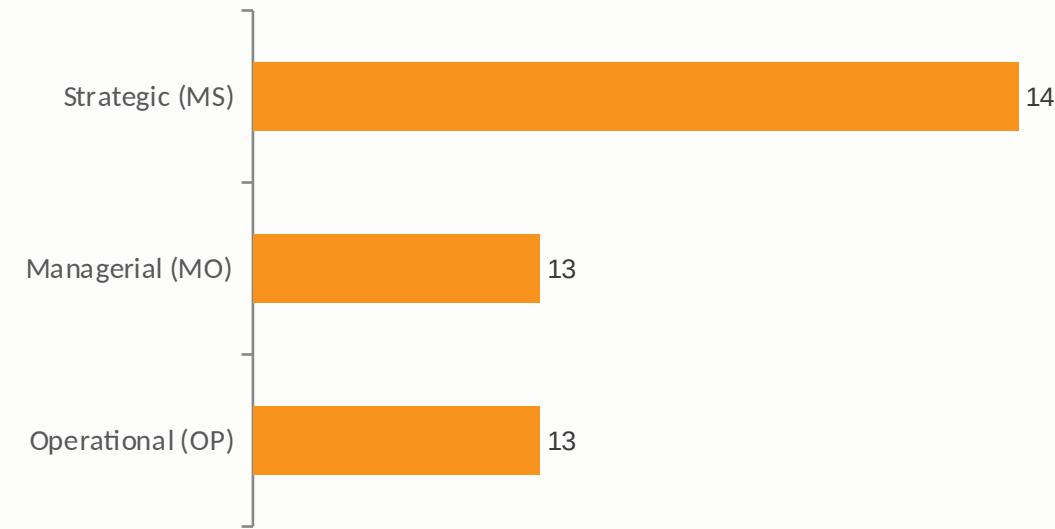
stakeholders in the register

**40/40**

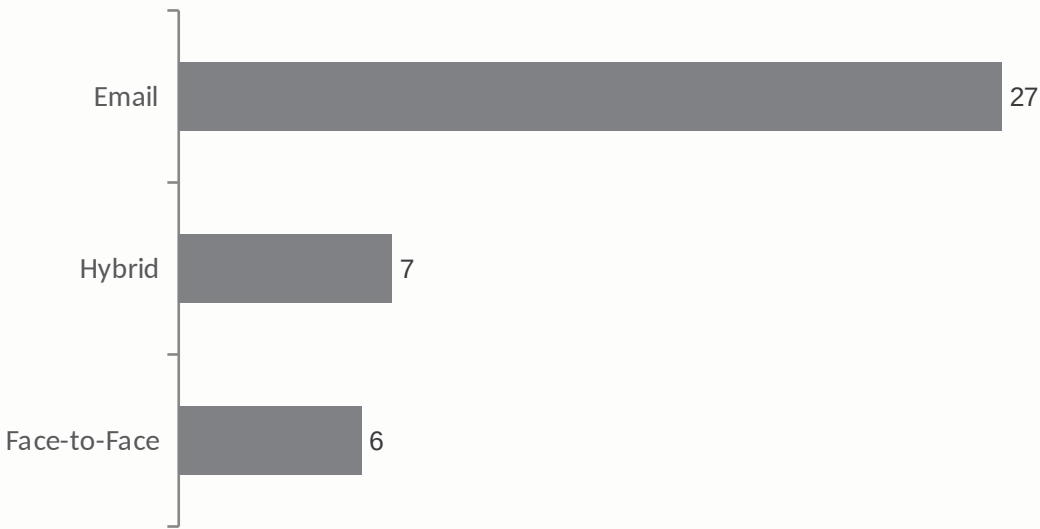
stakeholders with consent recorded

**v1**

approved version of the Scope of Work



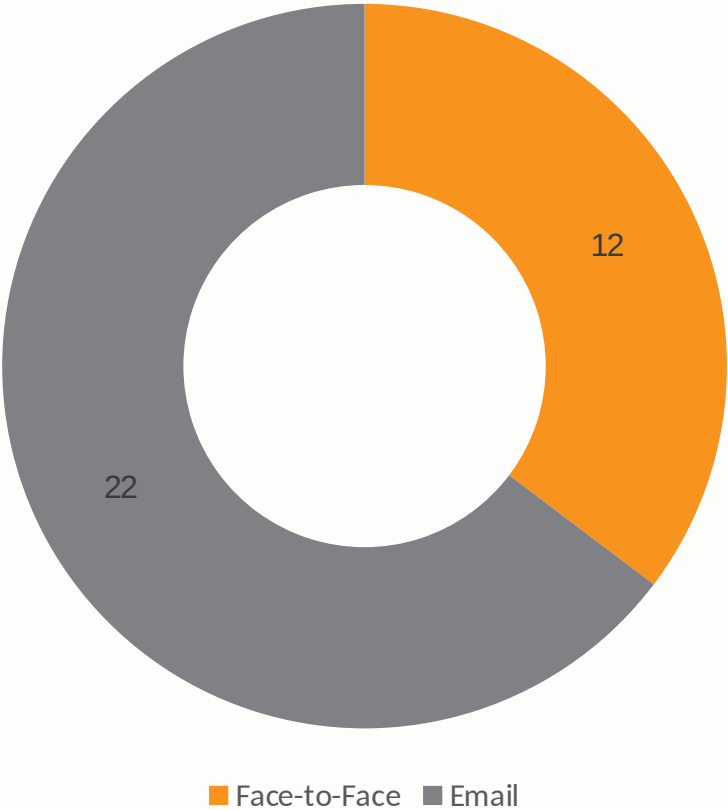
Stakeholders by decision level



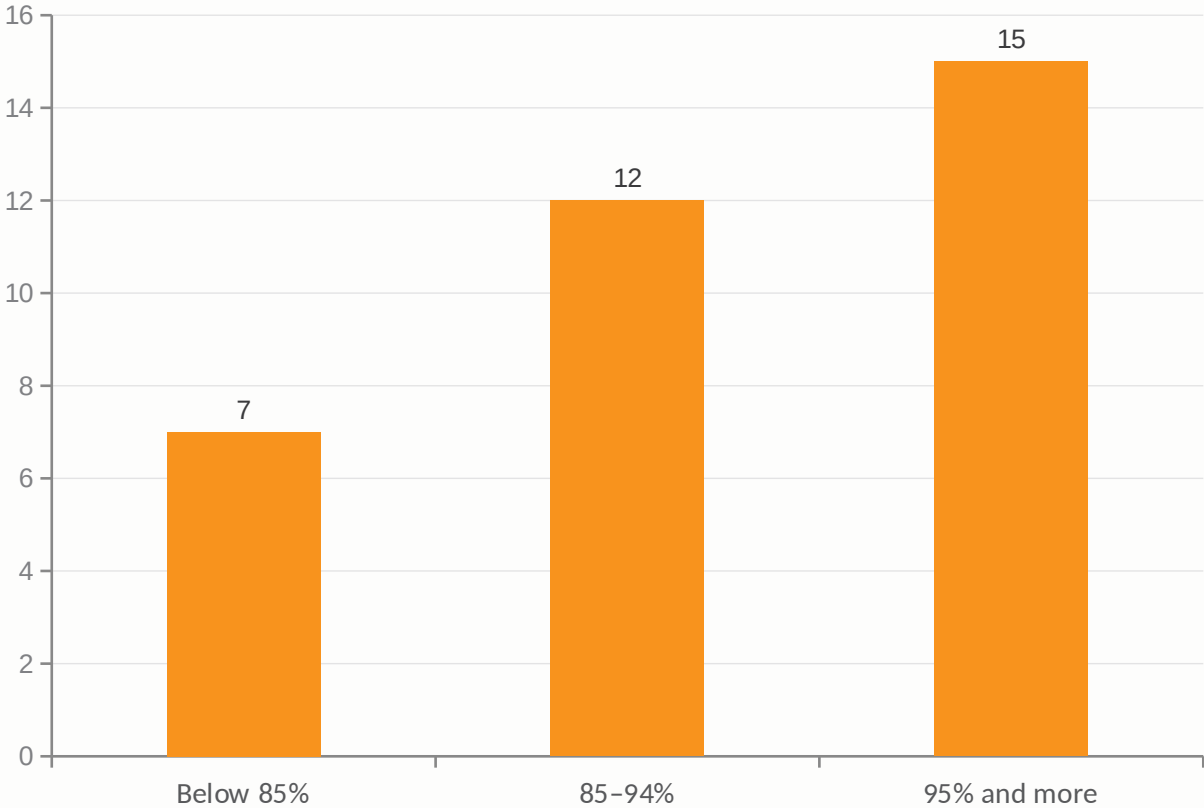
Stakeholders by preferred channel

34 questionnaire responses, 91% complete on average

Questions drafted by AI from the Scope of Work, validated by a person, sent through each stakeholder’s channel.



Questionnaire responses by channel used



Responses by completeness of the answers

G1 — Scope & evidence approved: “Go”

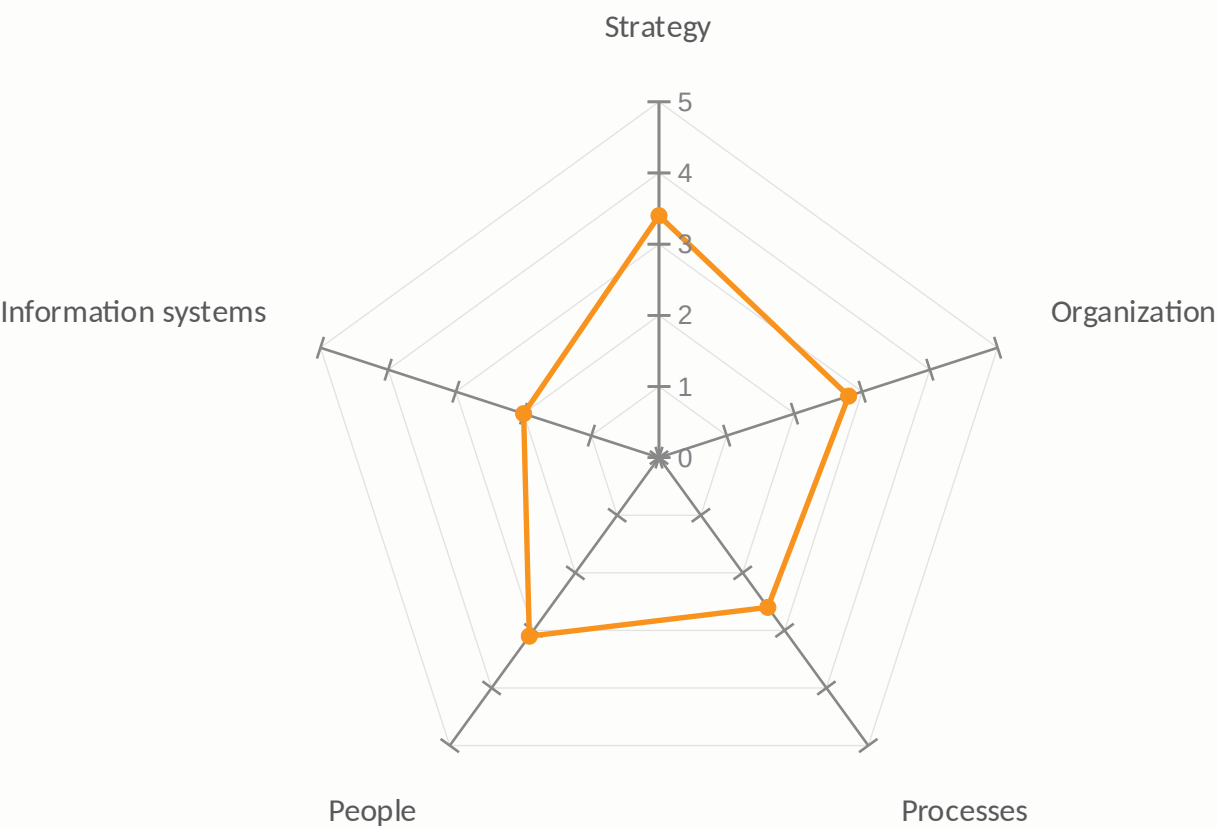
Decided on 23 Jul 2026 · All mandatory items met; evidence attached.

Checklist item	Mandatory	Evidence	Status
Scope frame recorded (Group Yes/No, Organization, sector)	Yes	—	Done
Decision levels and functions mapped in the criteria matrix	Yes	—	Done
Stakeholder register complete with channels	Yes	Attached	Done
Every AI-generated question validated by a person	Yes	—	Done
Consent confirmed for every stored response	Yes	Attached	Done

A gate cannot be signed off while a mandatory item is open; the approvers are HR Director and Consultant PM.

Five-axis diagnostic: information systems score lowest

Findings validated with the sponsor • scores on a 0–5 scale



Score of each diagnostic axis (0–5)

CMM maturity

3 → 4

current level → target level (1–5)

Leadership maturity

3 → 4

current level → target level (1–5)

Lowest axis: information systems (2). Highest: strategy (3.4).

11 of 21 cells are Red

Performance by function and decision level: MS strategic, MO managerial, OP operational.

Function	MS	MO	OP
Operations	79%	67%	59%
Quality & HSE	88%	68%	87%
Finance	90%	73%	74%
Human Resources	57%	53%	65%
Sales & Marketing	54%	80%	56%
Information Systems	92%	54%	77%
Clinical Care & Nursing	63%	87%	59%

Score per cell; fill shows the RAG status (red, amber, green)

Clinical Care & Nursing

MS

63% • Red

Gap: AI triage and clinical decision support

MO

87% • Amber

Gap: AI medical imaging assistance

OP

59% • Red

Gap: AI bed and staff capacity planning

Four improvement axes, ranked

Functional SWOT validated for Operations and Quality & HSE.

Strengths

Experienced team; stable processes

Weaknesses

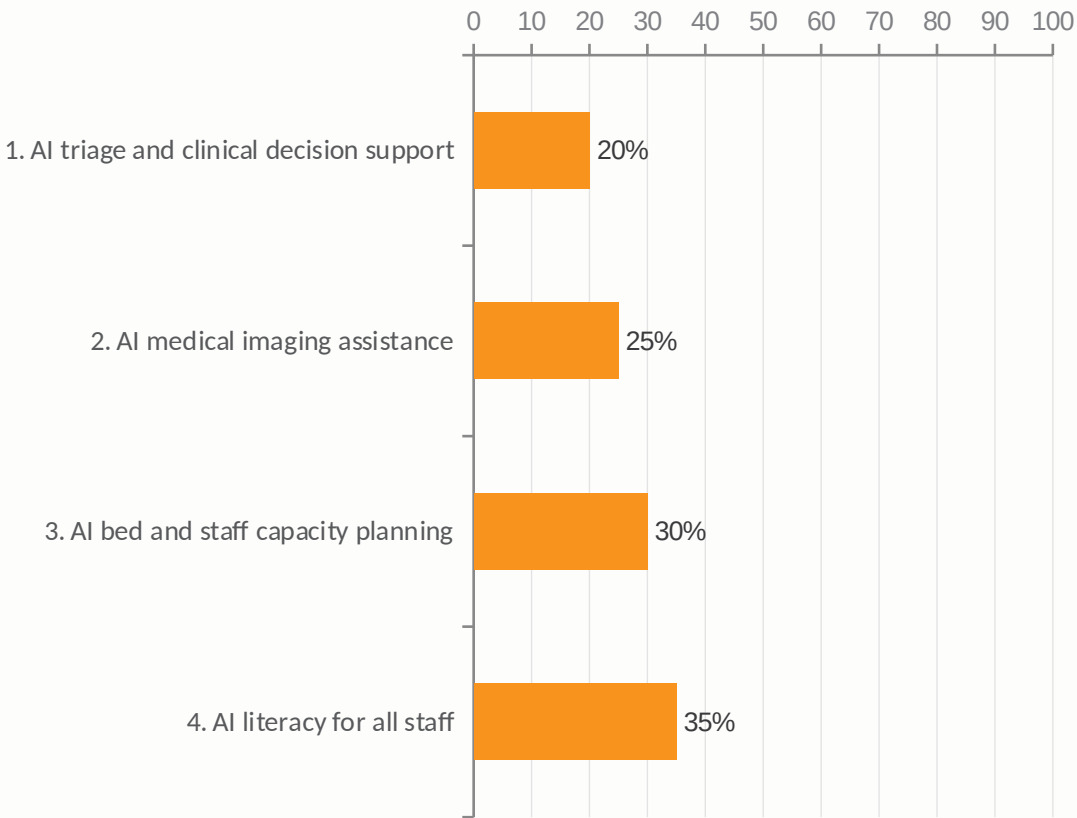
Low use of AI triage and clinical decision support

Opportunities

AI bed and staff capacity planning

Threats

Stricter ISO 7101 audits



Implementation progress of each improvement axis, by priority rank (%)

From individual gaps to validated training demands

Gaps come from skills assessments; needs and demands are validated before prioritization.

**12**

skills assessments recorded

**8**

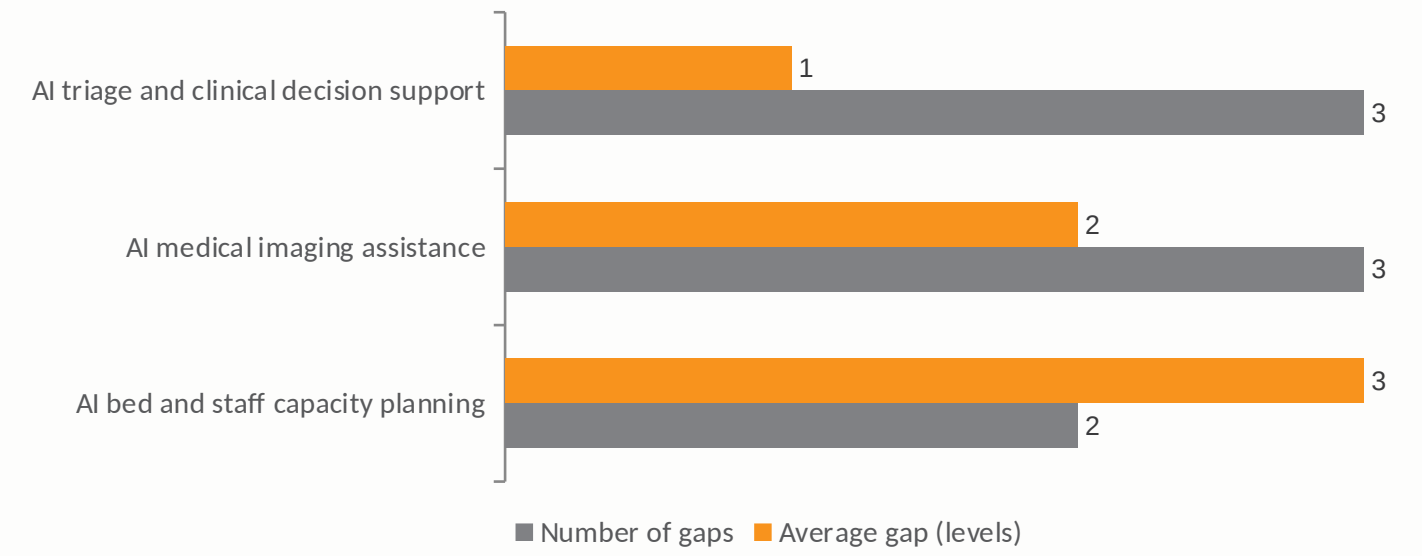
skill gaps, 2 critical

**10**

training needs, 10 validated

**15**

training demands from managers and staff



Skill gaps by competency: average gap in proficiency levels and number of gaps

Training demands by priority

Priority	Demands
High	5
Medium	5
Low	5

Six AI themes, ranked by impact and urgency

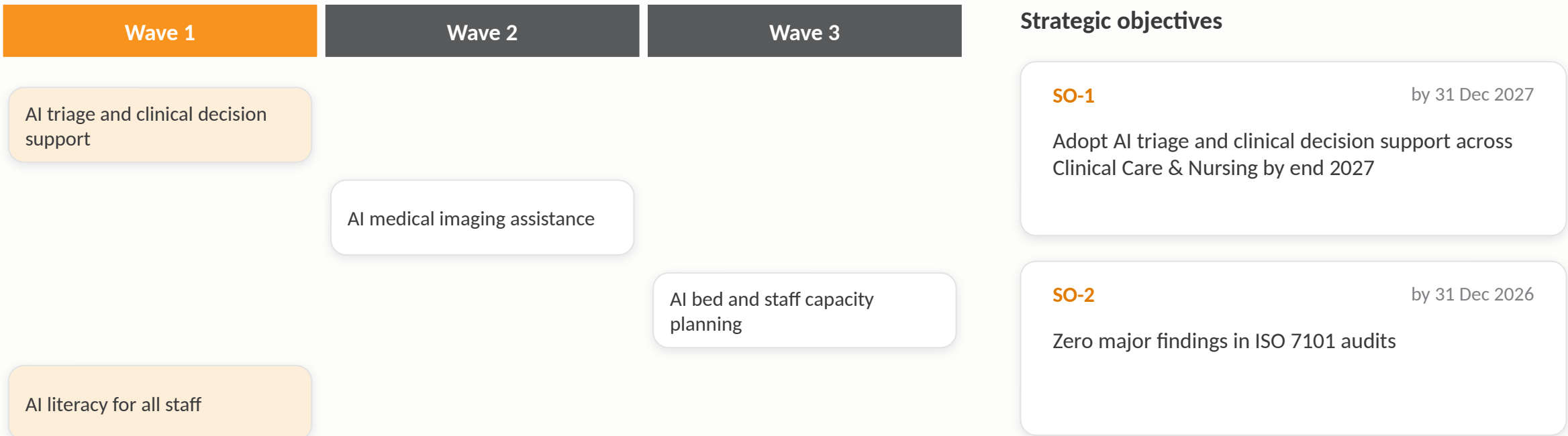
AI proposes a priority score; the L&D Strategist accepts or changes it with a justification.

#	Theme	Impact	Urgency	Days × groups	Trainer	Budget
1	AI triage and clinical decision support	5	4	3 × 6	Internal	360,000
2	AI medical imaging assistance	4	3	2 × 4	Vendor A	300,000
3	AI bed and staff capacity planning	3	4	2.5 × 3	Internal	240,000
4	AI literacy for all staff	5	3	1.5 × 3	Vendor A	180,000
5	Prompting and generative AI at work	4	4	1 × 2	Internal	120,000
6	Responsible AI and data protection	3	3	2 × 2	Vendor A	60,000

Impact and urgency on a 1-5 scale; cell fill follows the status scale (red = 1, dark green = 5). Budget as recorded in the plan.

The Training Engineering Report, published and versioned

Status Published • version 2 • 4 sections drafted by AI and reviewed by the Quality Manager

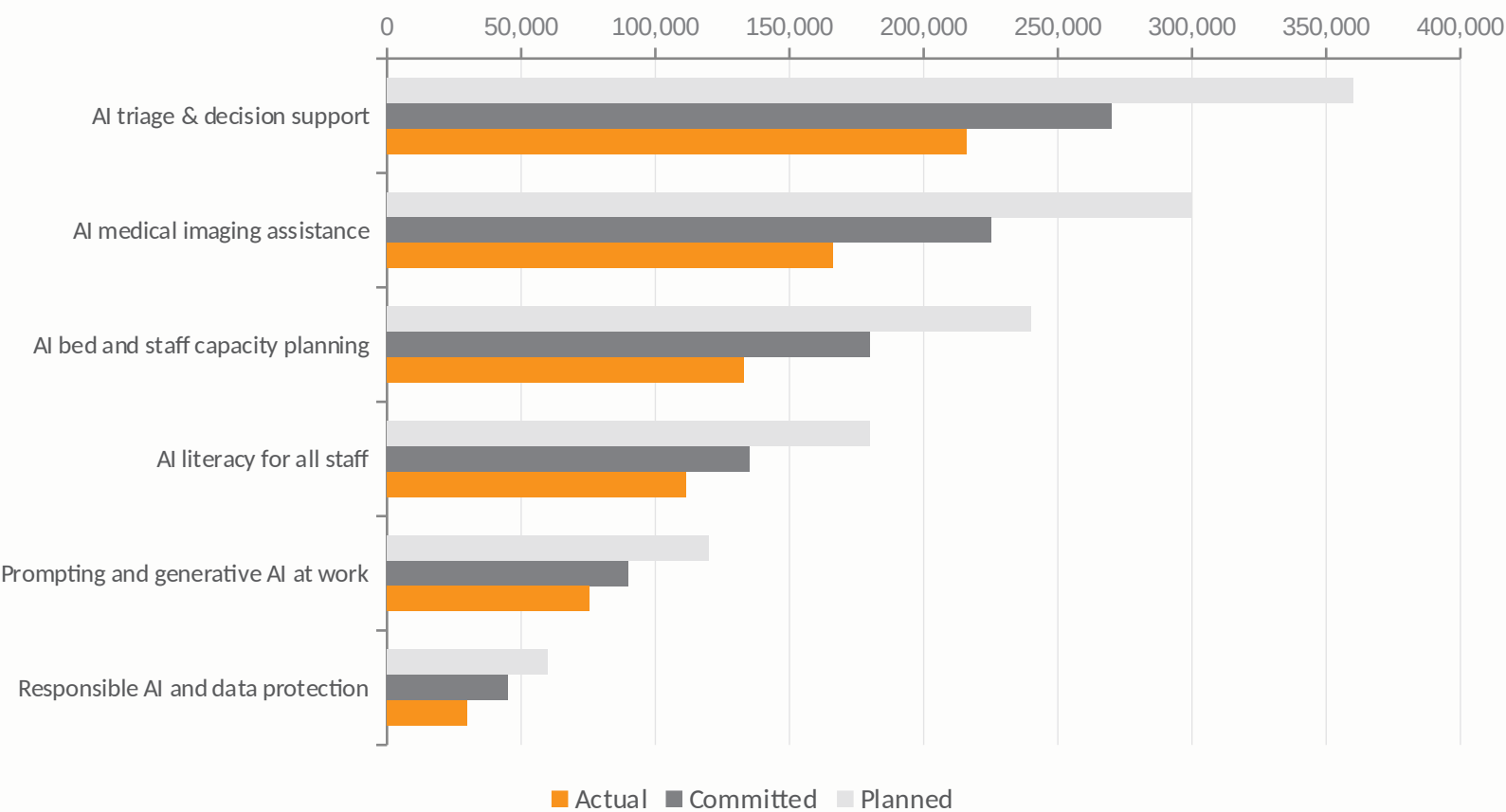


Roadmap initiatives by wave

Input: Healthcare Providers competitive intelligence 2026, updated 24 Sept 2026.

2026 plan locked: 1,260,000 budgeted, 35% realized

Planned, committed and actual amounts per theme, as recorded (currency set per organization).



Planned, committed and actual spend per theme



945,000
committed (75% of planned)



731,767
actual spend (58% of planned)



352,800
refund forecast from training contracts

Training contract refunds, tracked to the deadline

OFPPT special training contracts file • regulator OFPPT • deadline 23 Oct 2026 • status Approved

Theme	Planned	Actual	Refund forecast	Refund / planned
AI triage and clinical decision support	360,000	215,877	100,800	28%
AI medical imaging assistance	300,000	166,256	84,000	28%
AI bed and staff capacity planning	240,000	133,013	67,200	28%
AI literacy for all staff	180,000	111,516	50,400	28%
Prompting and generative AI at work	120,000	75,266	33,600	28%
Responsible AI and data protection	60,000	29,839	16,800	28%



Deadline alerts

A missed submission deadline means a lost refund. The run records the file, its deadline and its status, and the risk register tracks “Missed regulatory submission deadline and lost refund”.

Learning paths, modules and sessions

2 learning paths • 4 modules • 4 sessions • 12 enrollments

Module	Modality	Hours	Design status
M1 AI triage and clinical decision support	E-learning	2	Approved
M2 AI medical imaging assistance	Classroom	7	Approved
M3 AI triage and clinical decision support	Virtual	3.5	Draft
M4 AI medical imaging assistance	Classroom	7	Draft

Session	Starts	Capacity
S-01 AI triage and clinical decision support	3 Oct 2026	12
S-02 AI medical imaging assistance	10 Oct 2026	12
S-03 AI triage and clinical decision support	17 Oct 2026	12
S-04 AI medical imaging assistance	24 Oct 2026	12



4

content assets tagged to competencies



48

seats scheduled across the sessions

Attendance is recorded per session; a missing attendance record raises an alert to the Training Administrator.

Guidance written for this hospital and this sector

The task panel shows the expected answer, the inputs and outputs, the owner and the evaluator.

CortexSkills

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PROCESS DESIGN

Macro processes

End-to-end processes

Maghreb Hospitals Group

All projects

E2E-03 • ROOT / MASTER CHAIN (FLAGSHIP)

Needs-to-Training Engineering Report &
Produce the Training Engineering Report and the prioritized training roadmap

AI Skills Full Run 2026 — Maghreb Hospitals Group

Process specification

User-facing tasks

#	Task	Status	Owner
1	UFT-03-01 Plan diagnostic interviews	Completed	Hind Ouazzani
2	UFT-03-02 Record interview findings	Completed	Hind Ouazzani
3	UFT-03-03 Cluster needs into themes	Completed	Hind Ouazzani
4	UFT-03-04 Prioritize needs by impact	Completed	Tarik Amrani
5	UFT-03-05 Validate needs register	Completed	Mehdi Hajji
6	UFT-03-06 Consolidate engineering inputs	Completed	Hind Ouazzani
7	UFT-03-07 Draft Training Engineering Report	Completed	Tarik Amrani
8	UFT-03-08	Completed	Leila Sefrioui

UFT-03-04 • S03-04 • TASK

Prioritize needs by impact

Completed

Due: 1 Sept 2026

Score each need by business impact and urgency.

Work

RACSI

Files • 0

History

What to type

AI triage and clinical decision support: impact 5, urgency 4. AI medical imaging assistance: impact 4, urgency 3. ISO 7101: impact 4, urgency 5.

Input (supplier)

Training themes — L&D Strategist

Output (beneficiary)

Prioritized needs → CHRO

Steps

☒ MP-03.6 Prioritize needs by business impact

Score each need against strategic objectives, risk and population size. · L&D Strategist · User task

Your output

AI triage and clinical decision support: impact 5, urgency 4. AI medical imaging assistance: impact 4, urgency 3. ISO 7101: impact 4, urgency 5.

Owner

Tarik Amrani — L&D Strategist

Evaluator

Mehdi Hajji — Head of L&D

Must be a different person from the owner.

Reopen



“What to type” is specific to the organization, its sector and the focus of the run.



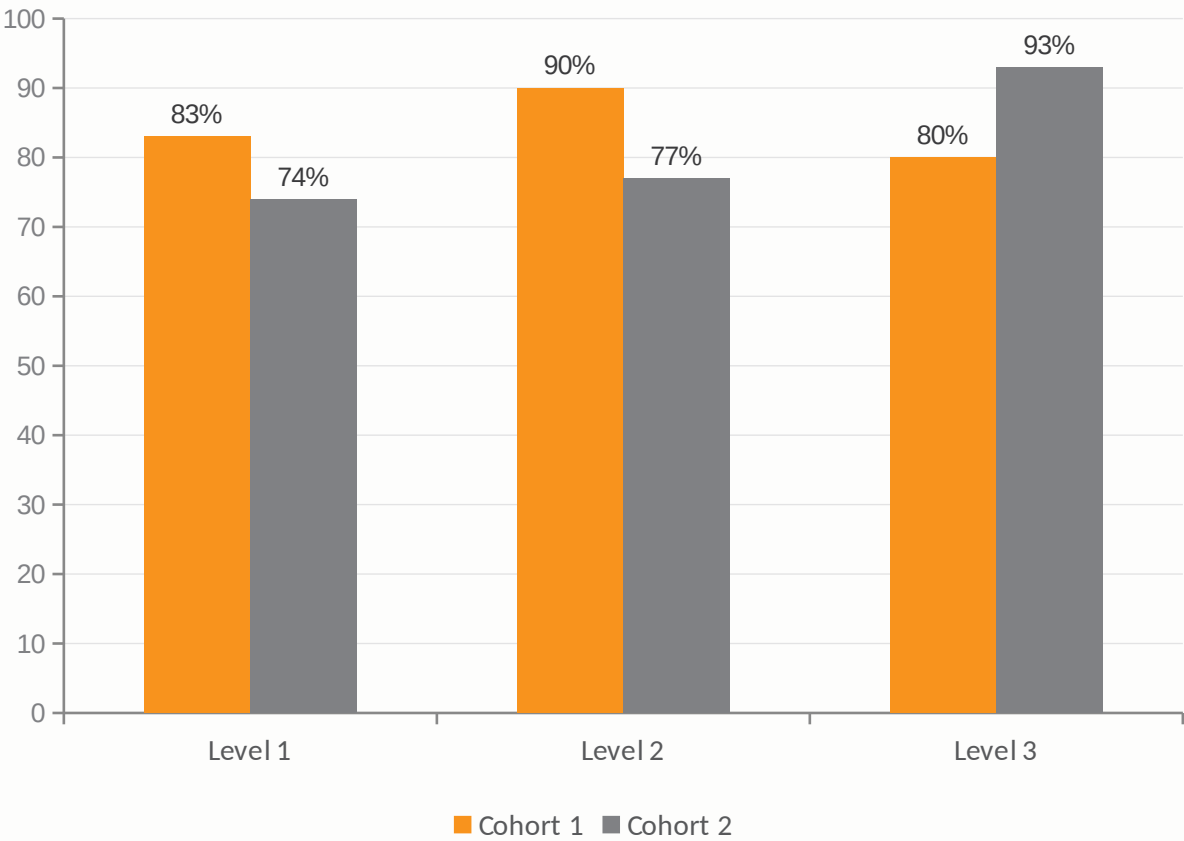
AI help proposes a draft, labelled with its confidence and sources.



Owner and evaluator are two different people: segregation of duties is enforced.

Kirkpatrick levels 1 to 3, and certificate validity

Evaluation results per enrollment; certificates tracked against their expiry date.



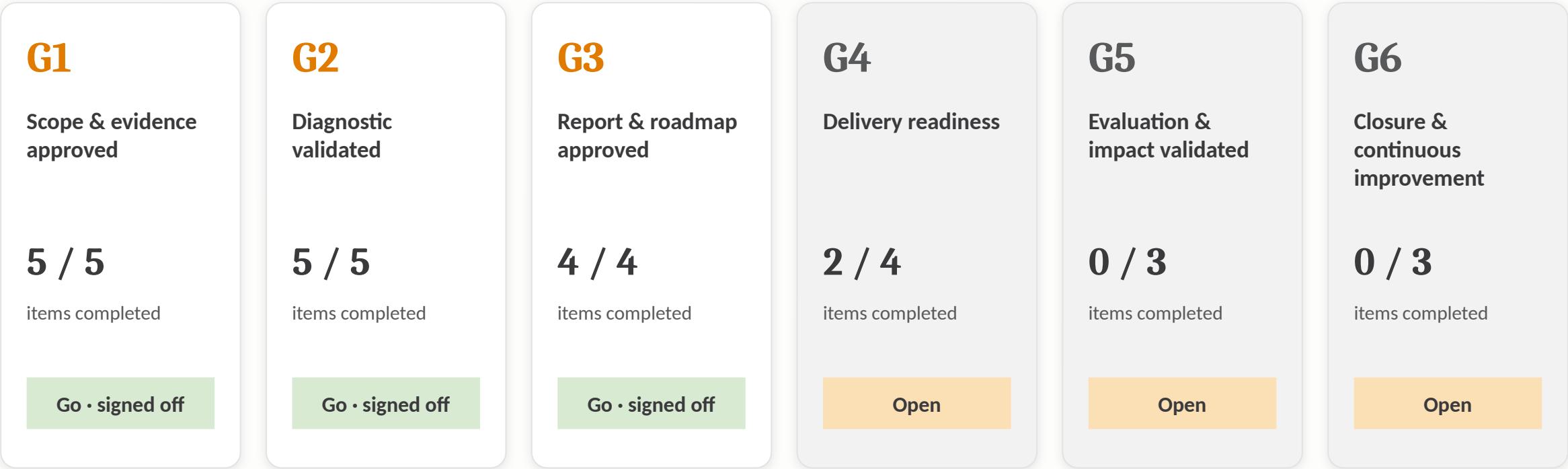
Evaluation score by Kirkpatrick level: 1 reaction, 2 learning, 3 behaviour (%)

Certificate	Valid until	Status
ISO 7101 awareness	10 Aug 2026	Valid
ISO 7101 awareness	8 Nov 2026	Expired
ISO 7101 awareness	6 Feb 2027	Valid
ISO 7101 awareness	7 May 2027	Expiring
ISO 7101 awareness	5 Aug 2027	Valid
ISO 7101 awareness	3 Nov 2027	Valid

4 valid • 1 expiring • 1 expired. Expiry raises an alert before the date.

3 gates signed off, 3 open

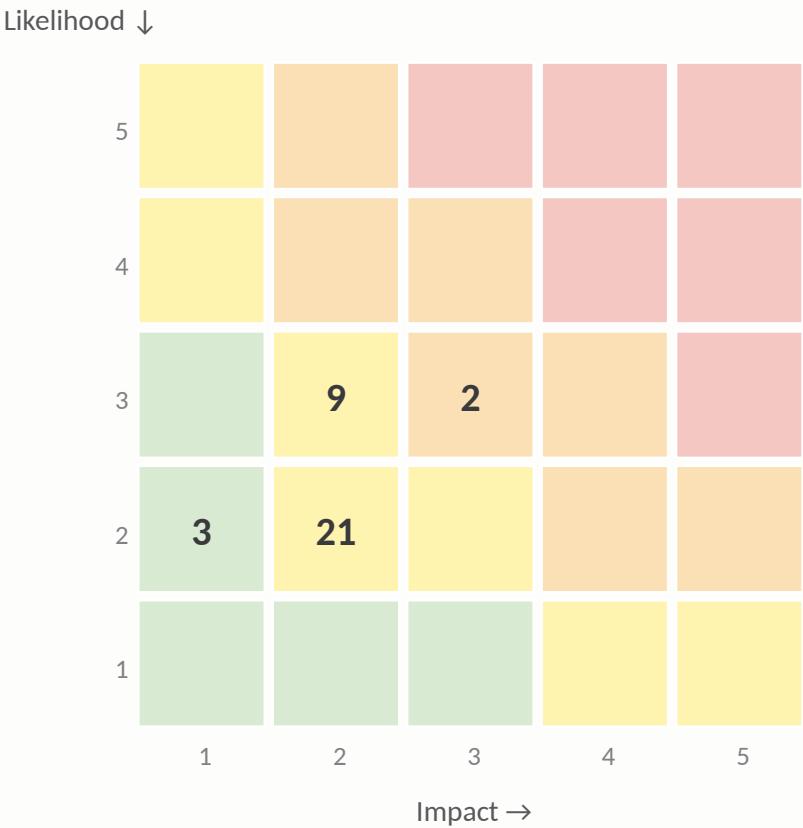
Where the run stands: mandatory items completed at each gate.



Each gate has its own checklist; the sign-off records the decision, the approver and the date.

35 risks scored by likelihood and impact

Each risk is linked to its controls, a key risk indicator and an owner.

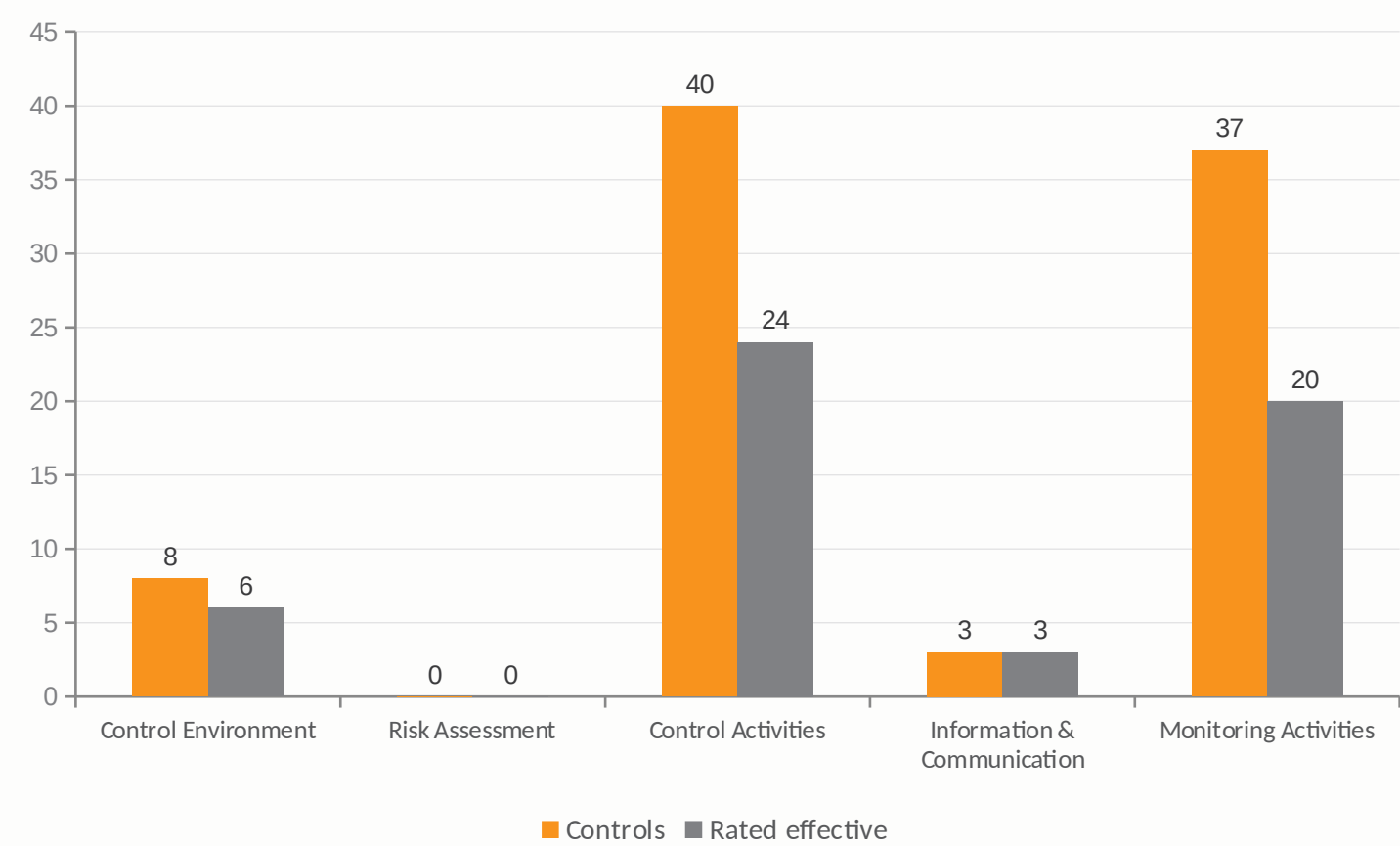


Code	Risk	Category	Score
RISK-014	Low learner and user adoption	Adoption	9
RISK-017	Critical knowledge lost with expert departure	Operational	9
RISK-001	Unauthorized access to tenant data	Security	6
RISK-005	Training plan misaligned with strategy	Strategic	6
RISK-006	Inaccurate skills data leading to wrong needs	Data quality	6
RISK-007	Unverified AI content in the Training Engineering Report	AI	6
RISK-009	Expired mandatory certification	Compliance	6

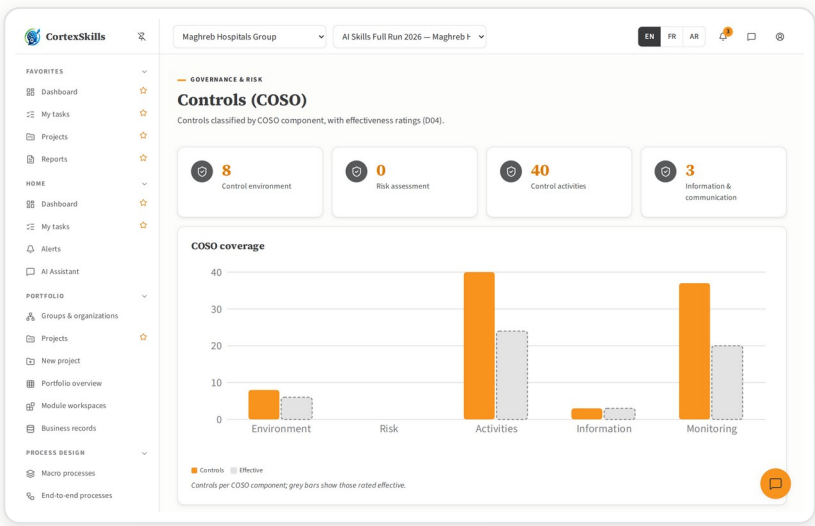
Number of risks per likelihood × impact cell

88 controls mapped to the COSO components

53 rated effective • 89 business rules enforce them in the workflow



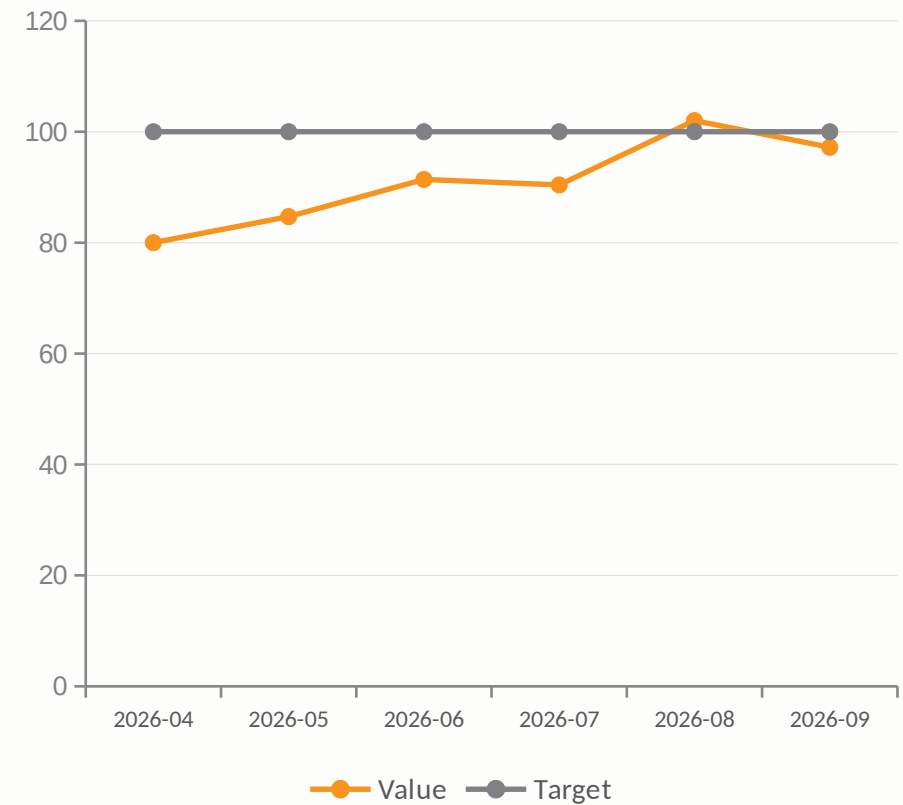
Controls and effective controls per COSO component



61 standard KPIs, measured monthly

Plus 2 custom KPIs, e.g. “Share of Clinical Care & Nursing supervisors certified”.

KPI	Latest	Target	Status
KPI-001 • Strategic objective coverage	97.2	100%	Amber
KPI-002 • Skills inventory validation rate	94.7	≥ 95%	Amber
KPI-003 • Critical gap rate	8.9	≤ 10%	Green
KPI-010 • Learner completion rate	82.9	≥ 80%	Green
KPI-020 • Integration synchronisation success rate	104.6	≥ 99.5%	Green
KPI-061 • Stakeholder engagement rate	72.9	≥ 75%	Amber



Strategic objective coverage: monthly value against target

15 open alerts, each with an escalation chain

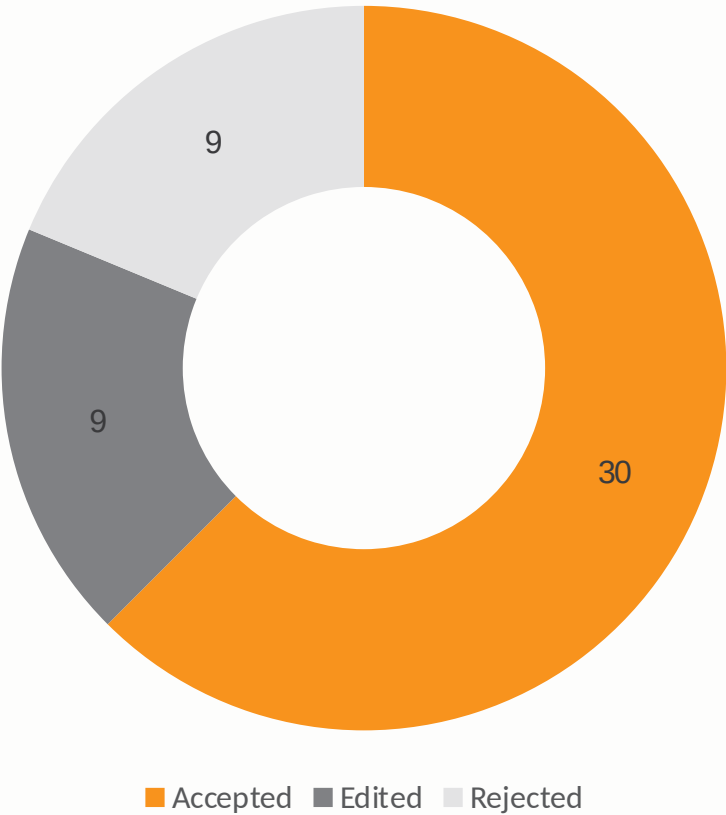
Alerts come from process rules; overdue steps escalate by role and by delay.

Alert	Step	Severity	Open	Escalation
ALR-006 • Record attendance	MP-07.5	Medium	1	Trainer → Training Administrator (24 h) → Head of L&D (72 h)
ALR-007 • Trigger re-engagement nudge	MP-08.6	Low	2	Learner → Manager (after second nudge)
ALR-008 • Review evaluation results	MP-09.7	Medium	2	L&D Analyst → Instructional Designer → Head of L&D
ALR-014 • Run HRIS synchronisation	MP-16.4	Medium	2	IT Architect → Platform Administrator
ALR-016 • Enforce login rate limit	MP-17.6	High	2	Platform Administrator → IT Architect
ALR-018 • Track adoption usage	MP-19.4	Medium	2	Change Manager → Head of L&D
ALR-020 • Execute improvement action	MP-20.3	High	2	Action Owner → Quality Manager (7 days) → Head of L&D
ALR-039 • Review access violation log	MP-17.4	Medium	2	IT Architect → Platform Administrator

Alerts, approvals and task notices are sent in the application and by e-mail (11 dispatches in the demonstration run).

48 AI suggestions logged: 30 accepted, 9 edited, 9 rejected

The usage log is append-only; each line keeps the use case, the user, the confidence and the decision.



AI suggestions by outcome of the human review

Maghreb Hospitals Group

AI Skills Full Run 2026 — Maghreb H

EN

FR

AR

FAVORITES

Dashboard

My tasks

Projects

Reports

HOME

Dashboard

My tasks

Alerts

AI Assistant

PORTFOLIO

Groups & organizations

Projects

New project

Portfolio overview

Module workspaces

Business records

PROCESS DESIGN

Macro processes

End-to-end processes

AI & KNOWLEDGE

AI usage log

Append-only log of every suggestion and assistant answer.

30

Accepted

9

Edited

9

Rejected

0

Assistant answers

Q Search

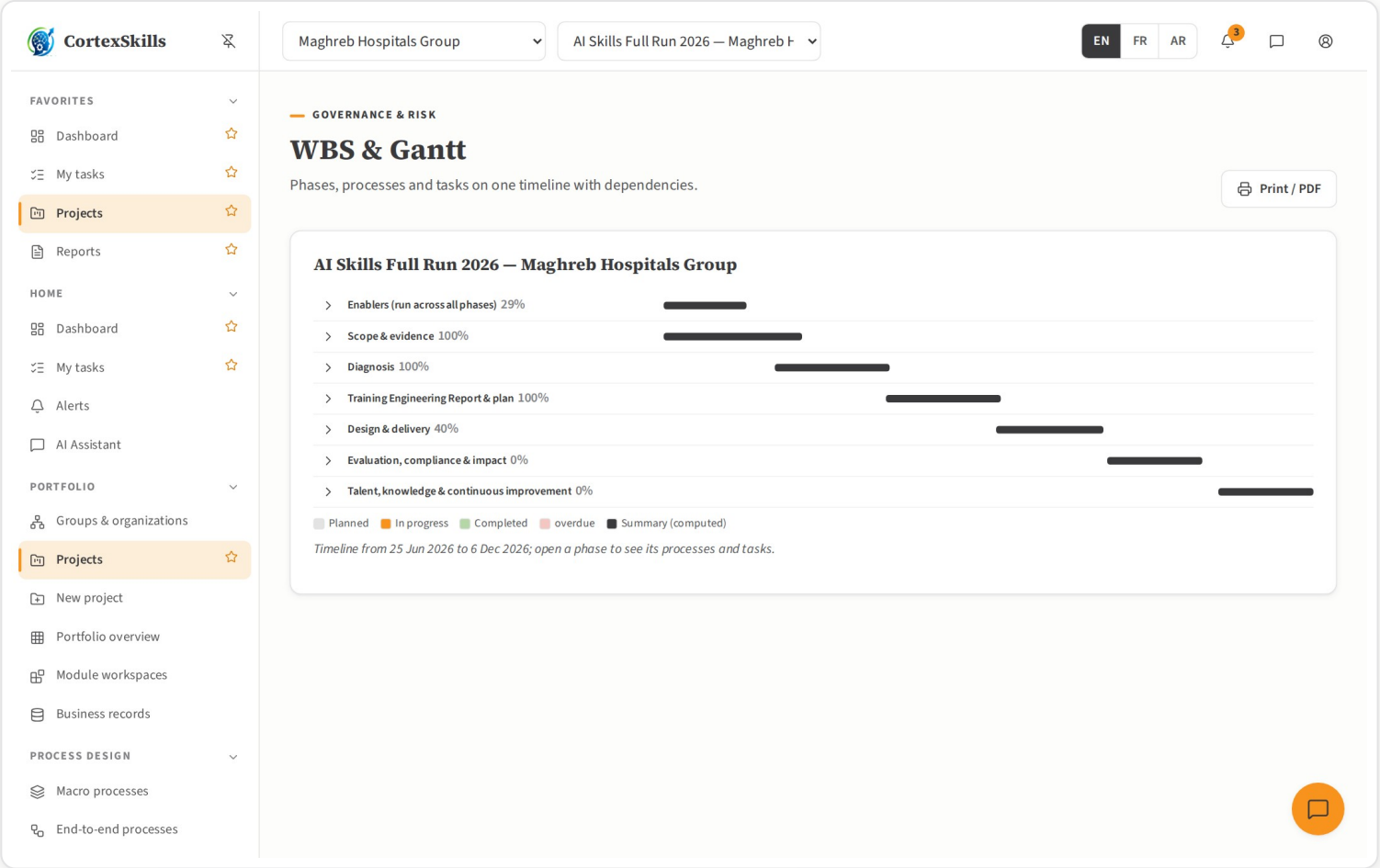
48 rows

CSV

Date	Use case	Outcome	Source	Confidence	User	Question or record
1 Oct 2026	AIUC-14	Approved	built-in	70%	Hind Mansouri	09fa8c2a-f5c8-5d93-a109-647b30a9013e
25 Sept 2026	AIUC-13	Approved	built-in	70%	Hind Mansouri	22809d0c-c653-5861-ae5d-bd3243443450
22 Sept 2026	AIUC-17	Rejected	built-in	70%	Mehdi Hajji	c93149e3-a482-5427-aae7-1aaeb5893d10
22 Sept 2026	AIUC-06	In review	built-in	70%	Zineb Squalli	e74345ee-190d-5e1e-a56f-b6d1f094c92b
22 Sept 2026	AIUC-07	Approved	built-in	70%	Hind Mansouri	09cbae5e-d828-5c02-a7c3-635ddc2de1d8
12 Sept 2026	AIUC-16	Rejected	built-in	70%	Hind Ouazzani	7c413587-2de1-5ca6-a5ee-059b8846b5bb
9 Sept 2026	AIUC-03	Approved	built-in	70%	Tarik Amrani	3b2f6f2a-f6c0-5fd7-a5da-b42940586e8f
9 Sept 2026	AIUC-12	Rejected	built-in	70%	Hind Mansouri	fe50f01f-fb1d-5bfd-a27d-fdbc539d52e8
6 Sept 2026	AIUC-05	Approved	built-in	70%	Meryem Mansouri	4552b43b-d329-5b1b-a44c-e4439df5ca1e

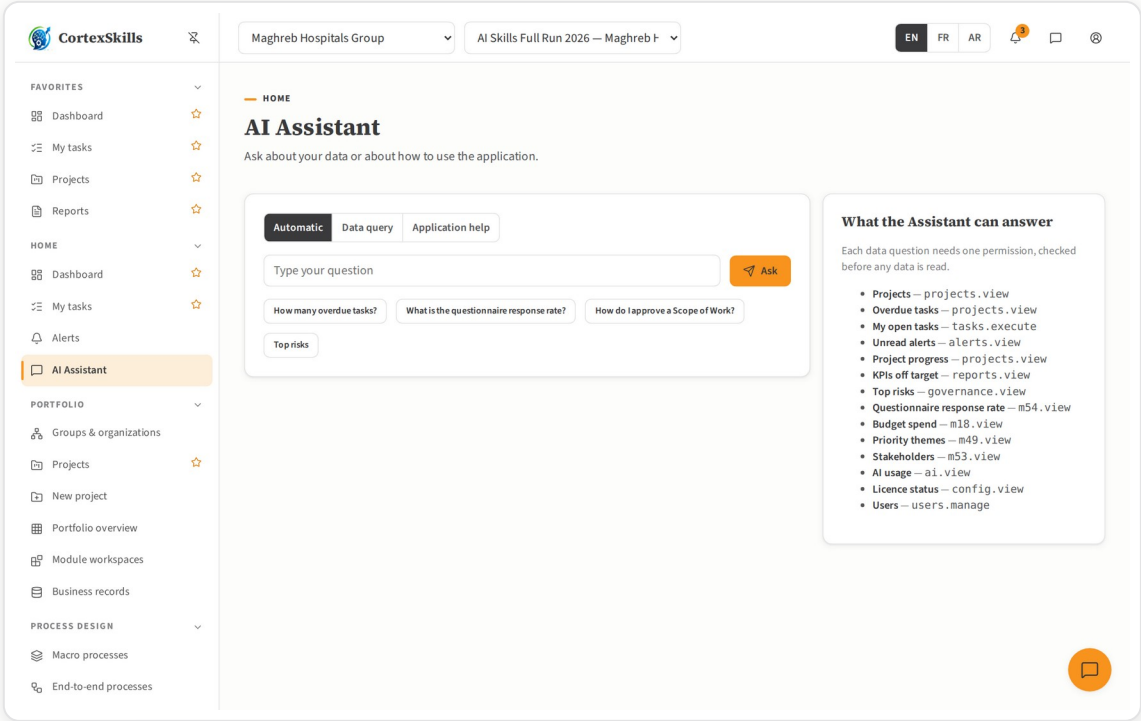
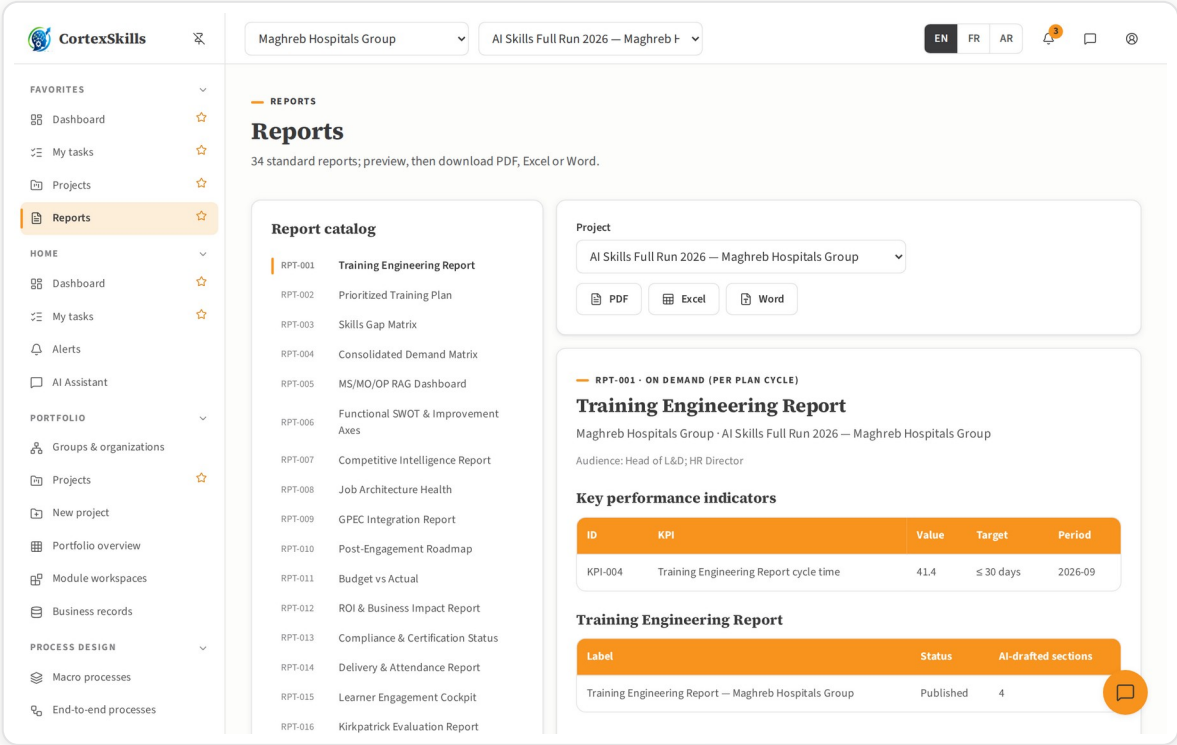
The whole run on one timeline

Phases, processes and tasks with dependencies; summary bars are computed from the tasks.



34 standard reports and a data-aware assistant

Reports export to PDF, Excel and Word. The assistant answers within the user’s permissions.



Lessons learned feed the next cycle

Return on experience rated 4/5 • 4 knowledge base articles for the sector standards

What went well	The stakeholder register and multi-channel distribution lifted the response rate.
What did not	SWOT inputs arrived late from one function.
Root cause	No backup nominated for the function head.
Recommendation	Name a backup for every function head at kickoff.

Knowledge base
ISO 7101 — training and competence requirements
JCI Standards — practitioner notes
GPEC — Forward Planning of Jobs and Skills
Law 09-08 — personal data in questionnaires

04

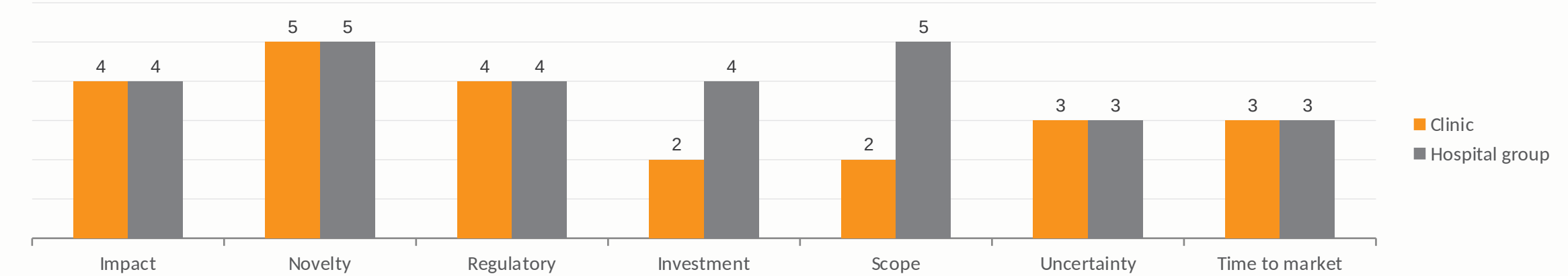
Case study: a clinic

Clinique Al Amal — 95 employees, Marrakech. The same method on a lighter track.

Complexity score 66: SME track T3

Clinique Al Amal • 95 employees • 11 users • Law 09-08 data protection

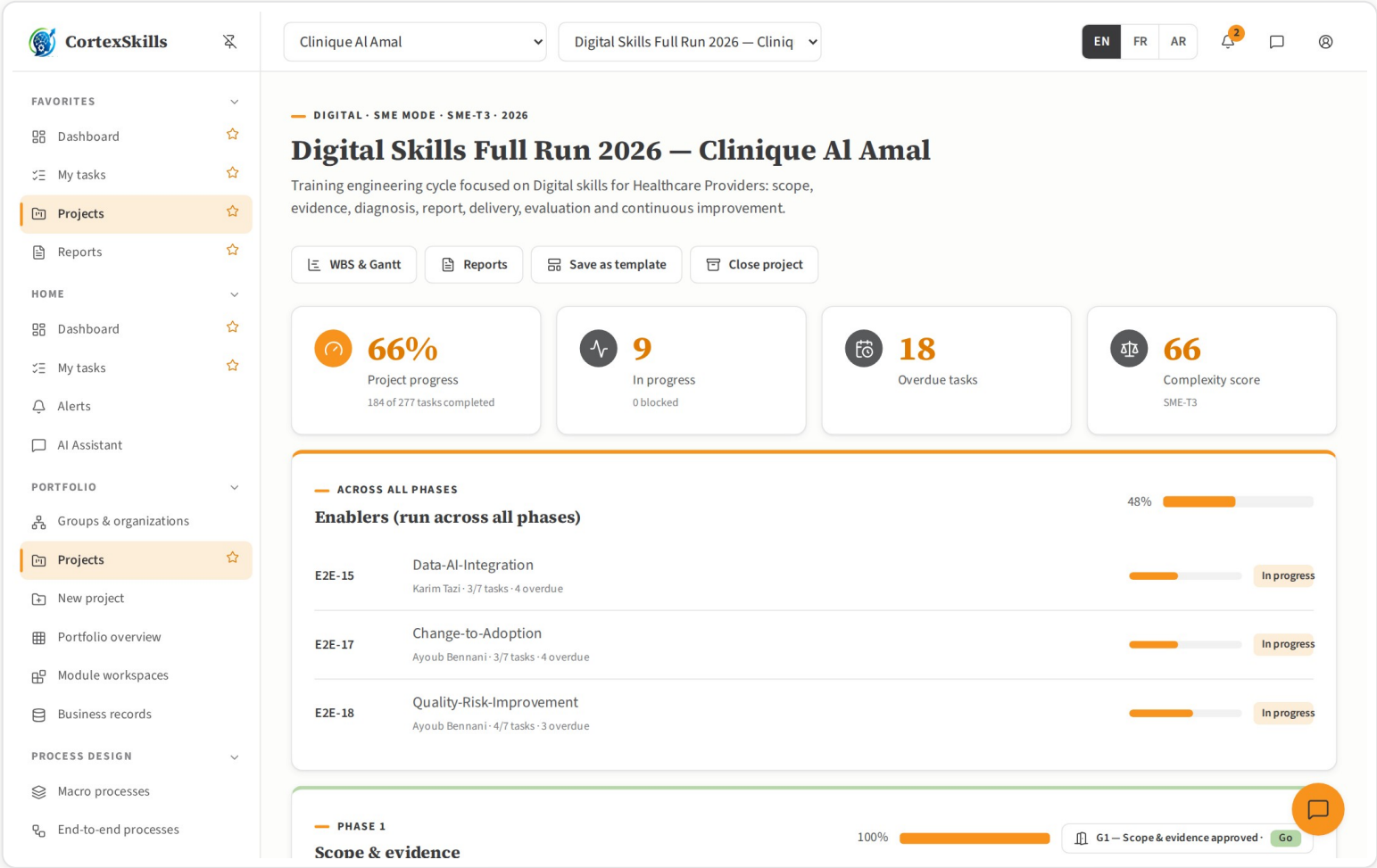
Track	For	Gates	Items per gate	Duration	Score
SME-T1 • Essential track	Micro	G1 • G3 • G6	3	about 30 days	0-35
SME-T2 • Standard track	Small	G1 • G3 • G5 • G6	4	about 60 days	35-60
SME-T3 • Advanced track	Medium	G1 • G2 • G3 • G4 • G5 • G6	5	about 90 days	60-100



Complexity ratings (1-5), clinic compared with the hospital group: investment and scope drive the difference

Same lifecycle, fewer items to complete

AI Skills Full Run 2026 — Clinique AI Amal • 50% of 277 tasks completed



Digital run: 66% of 277 tasks completed; AI run: 50%.



Six gates with five items each on track T3, instead of the Full-mode checklists.



The recommended track is kept; another choice needs a written justification.

The same guidance in English, French and Arabic

The task panel on a desktop, the workspace in Arabic on a phone (right-to-left layout).

CortexSkills

FAVORITES

Dashboard

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AI Assistant

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New project

Portfolio overview

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PROCESS DESIGN

Macro processes

End-to-end processes

Clinique Al Amal

Digital Skills Full Run 2026 — Cliniq

E2E-15 • ROOT / ENABLER

Data-AI-Integration

Govern data, AI, integrations

User-facing tasks

#	Task	Status	Owner
1	UFT-15-01 Register external integration	Completed	Anas Amrani
2	UFT-15-02 Map fields and check health	Completed	Karim Tazi
3	UFT-15-03 Run HRIS synchronisation	Completed	Karim Tazi
4	UFT-15-04 Set data retention rule	Completed	Hajar Tazi
5	UFT-15-05 Maintain AI use case register	In progress	Anas Amrani
6	UFT-15-06 Set AI activation per scope	Not started	Anas Amrani
7	UFT-15-07 Review AI usage log	Not started	Karim Tazi

UFT-15-05 • S15-05 • TASK

Maintain AI use case register

In progress Due: 7 Aug 2026

Maintain and version AI use cases.

Work RACSI Files 0 History

What to type

Add custom use case 'Shift-report summarizer' (Assistive), version 1.

Input (supplier) Retention rules — Platform Administrator

Output (beneficiary) AI register → Head of L&D

Steps

☒ MP-16.7 Maintain AI use case register

Maintain purpose, data used, owner and version of each AI use case. - Platform Administrator -

User task

Your output

Add custom use case 'Shift-report summarizer' (Assistive), version 1.

Owner

Anas Amrani — Platform Admi

Evaluator

Hajar Tazi — HR Director / Stee

Must be a different person from the owner.

Save Mark blocked Complete

AR

مصححة الأمل

SME-T3 • 2026 • نطق المقابلة

المسار الكامل لكفاءات الرقمي

مصححة الأمل — 2026

دورة هندسة التكوين المتمحورة حول كفاءات الرقمي لقطاع مؤسسات تقديم الرعاية الصحية: النطاق والأدلة والتشخيص والتقرير والتنفيذ والتقييم والتحسين المستمر.

التقارير

تجزئة العمل وجانت

إغلاق المشروع

حفظ كنموذج

66%

تقدم المشروع

184 مهمة منجزة من أصل 277

9

قيد الإنجاز

0 متوقفة

18

المهام المتأخرة

One method, sized to the organization

Comparison of the two healthcare providers in the demonstration instance.

	Maghreb Hospitals Group	Clinique Al Amal
Employees • city	4,700 • Casablanca	95 • Marrakech
Complexity score • mode	84 • Full mode	66 • SME track T3
Tasks per run	265	277
AI run completed	65%	50%
Digital run completed	78%	66%
Users	29	11
Solution	BND-05 • 600 seats	SME-CMP • 100 seats
Data protection	GDPR, LAW0908, ISO9001	LAW0908



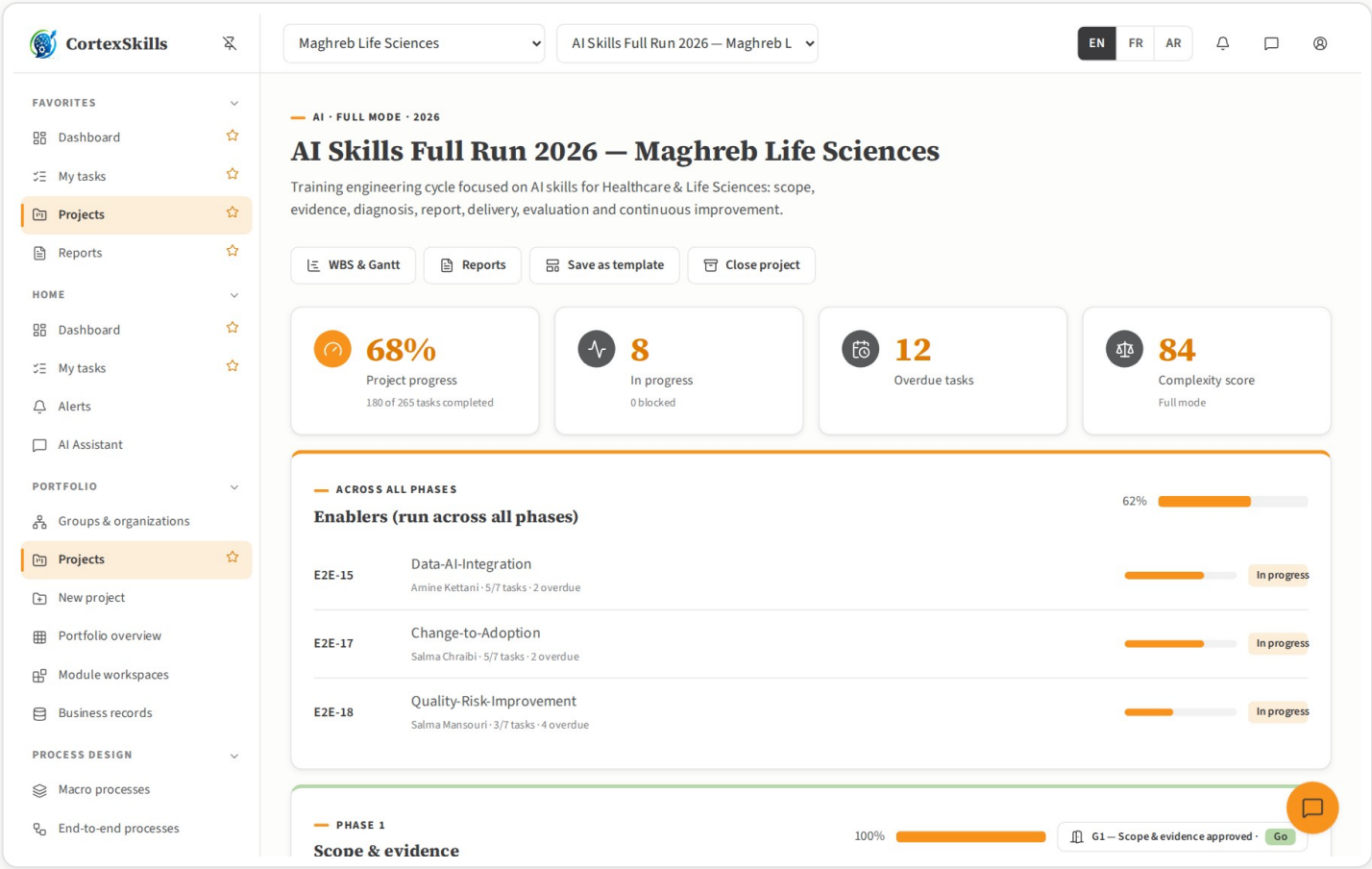
05

The health ecosystem

Life sciences, pharma distribution and pharmaceutical manufacturing

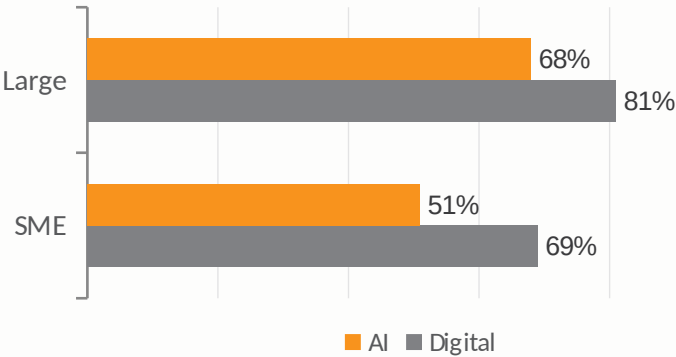
Healthcare & Life Sciences

Core function: Clinical & Laboratory Operations • ISO 13485, EU MDR, ISO 15189



Large company — Maghreb Life Sciences (3100)

SME — BioLab Diagnostics Rabat (60)



Task completion by run (%)



Priority AI themes

- AI-assisted diagnostics validation
- AI for pharmacovigilance signal detection
- Responsible AI and patient-data ethics

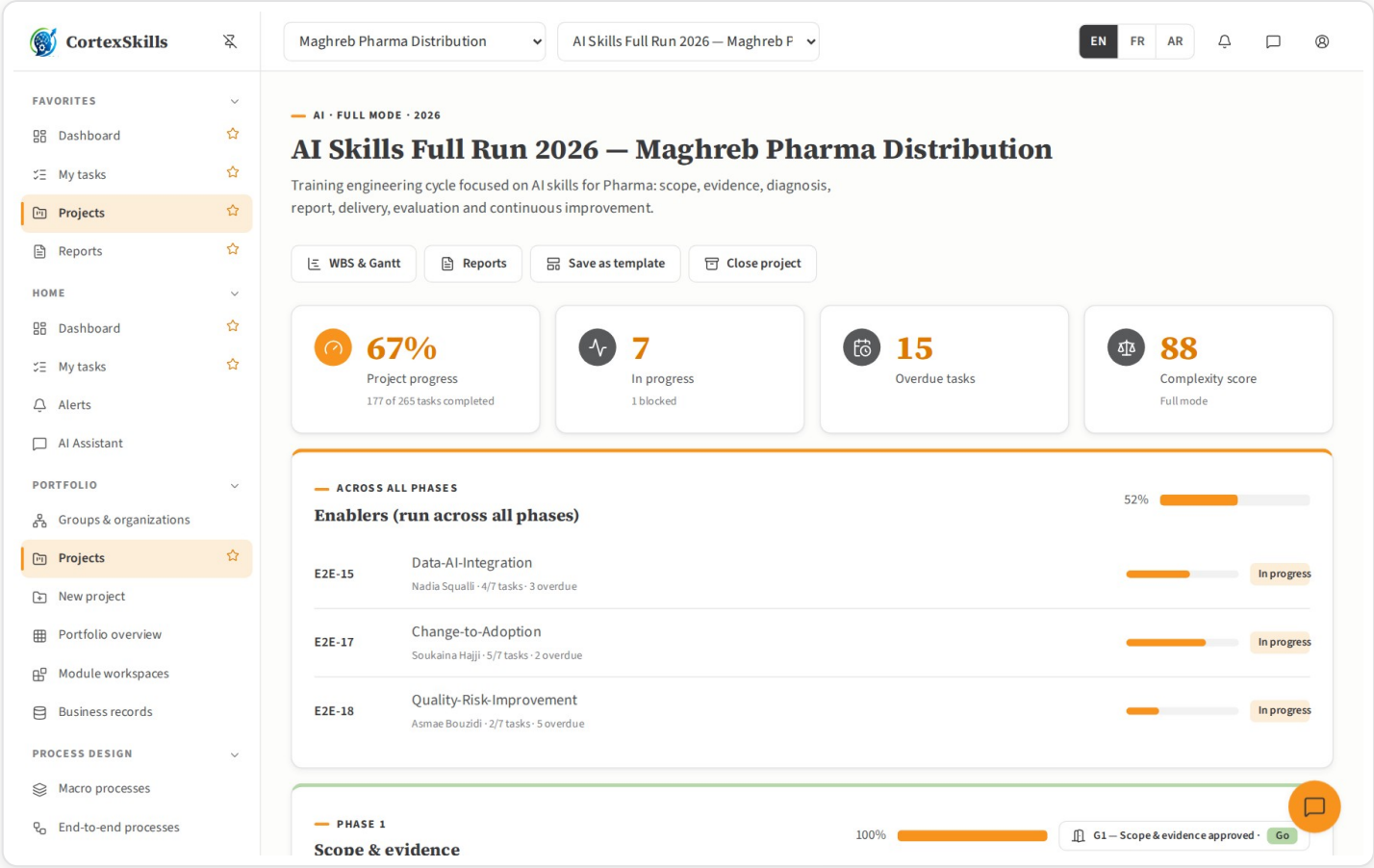


Priority digital themes

- Electronic lab notebooks and LIMS
- Electronic quality management (eQMS)
- Health data interoperability (HL7 FHIR)

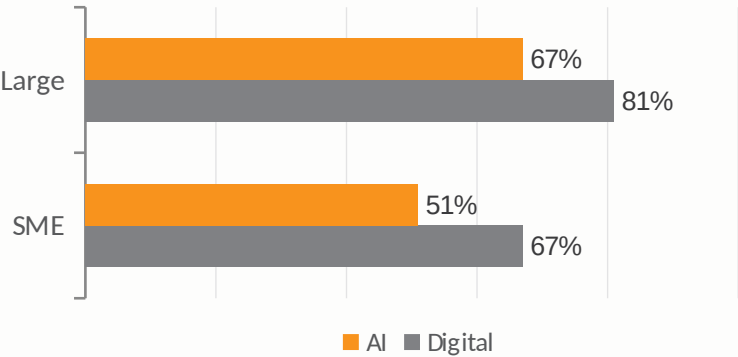
Pharma

Core function: Pharmaceutical Quality & Distribution • EU GMP, GDP, ICH Q9



Large company — Maghreb Pharma Distribution (1400)

SME — PharmaLog Services (44)



Task completion by run (%)



Priority AI themes

- AI regulatory-dossier drafting assistance
- AI demand sensing for medicines
- AI quality-risk management (ICH Q9)

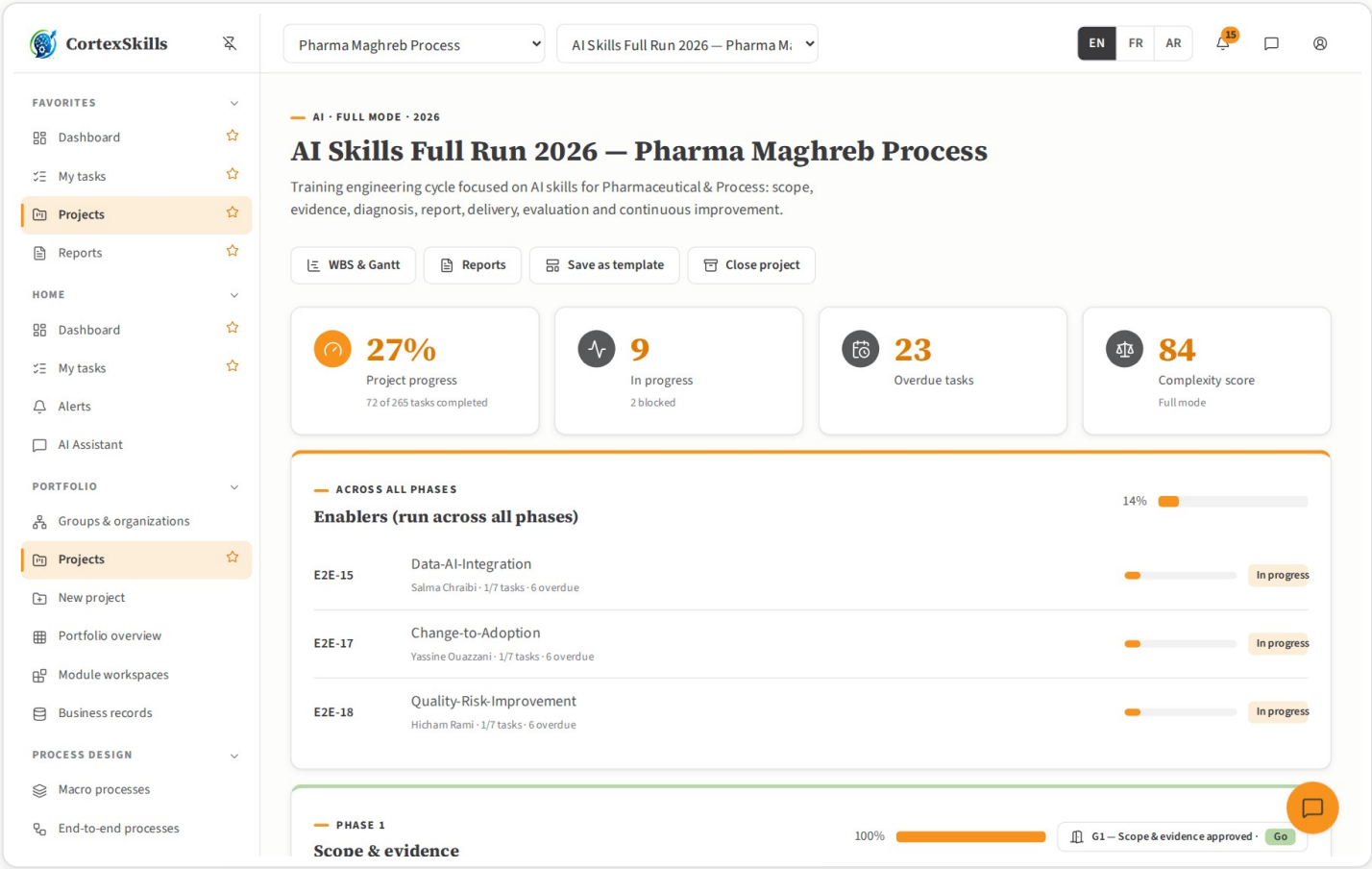


Priority digital themes

- Serialization and track-and-trace
- Digital GDP temperature monitoring
- Regulatory information management (RIM)

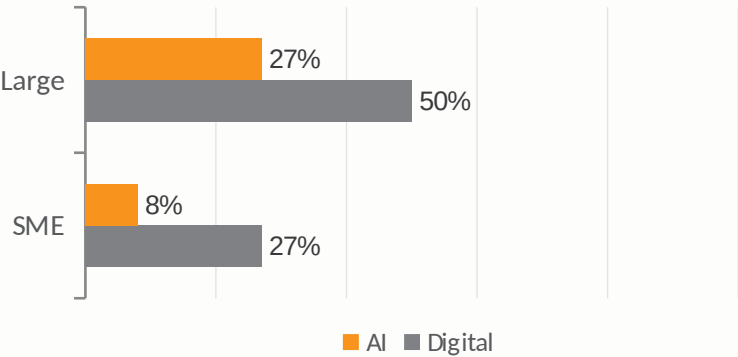
Pharmaceutical & Process

Core function: GMP Manufacturing • EU GMP, ICH Q10, FDA 21 CFR Part 11



Large company — Pharma Maghreb Process (2200)

SME — Galénix Laboratoires (90)



Task completion by run (%)

Priority AI themes

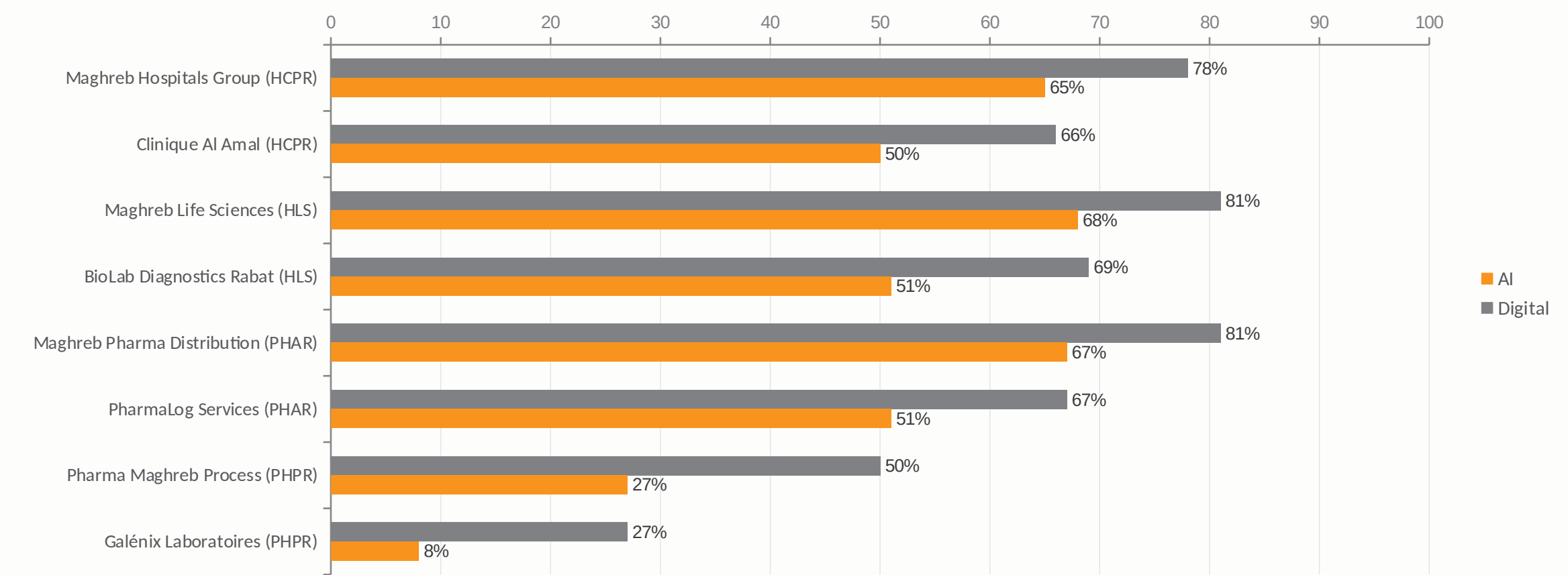
- AI process analytics for yield
- AI deviation and CAPA triage
- Validation of AI models in GxP

Priority digital themes

- Electronic batch records (EBR)
- Computer system validation (CSV)
- Data integrity and ALCOA+ practices

Eight health organizations, two runs each

Share of tasks completed in the Digital and AI runs of each demonstration organization.



Tasks completed in each run (%). Group members see these aggregates; records of sister organizations stay read-only.

06

Trust and next steps

Security, data protection, deployment and how to start

Patient-adjacent data, handled with strict boundaries

Staff data and training records stay inside the organization's tenant.



Tenant isolation

Each organization is a closed tenant; group members read sister organizations but cannot change them.



Role-based access

Permissions checked on the server for each of the 29 roles; pack entitlements enforced.



Audit trail

Every change to a governed field is justified and recorded in an append-only log.



Data-protection frameworks

GDPR, ISO/IEC 27001, SOC 2, ISO/IEC 27701, ISO 9001, Law 09-08 (CNDP, Morocco).



Governed AI

Each AI use case names its human checkpoint; the usage log keeps every outcome.



Consent

Consent recorded for every stored questionnaire response before gate G1.

18 integrations in the catalog

Three connections at the hospital group, each health-checked; the single sign-on check currently reports a failure.

Family	Integrations
Enterprise Systems	3
Talent & Performance Management	4
Learning Content & Ecosystem	4
Learning in the Flow of Work	2
Data & Analytics	2
Core Platform & Security	2
Survey & Feedback	1

Connected at the hospital group	Code	Last health check
HRIS / HCM	INT-HR-01	Success
Survey platform	INT-SUR-01	Success
SSO (Microsoft Entra ID)	INT-SSO-01	Auth failure

HRIS / HCM examples: Workday, SAP SuccessFactors, Oracle HCM, ADP, BambooHR, Personio, DarwinBox

SaaS or on-premises, same application

One licensing interface with two implementations, chosen when the server starts.



SaaS

- Hosted and updated centrally.
- Licence checked online per organization.
- Suited to clinics and groups without a local IT team.



On-premises

- Installed in the hospital's data centre.
- Signed licence file bound to the server, with expiry date and seat limit.
- Suited to strict data-residency policies.

Demonstration licence of the hospital group: plan BND-05, up to 600 users, features analytics, export, api, valid until 3 Nov 2027.

Recommended packs for a hospital or clinic

Catalog prices per month, in USD; each pack includes a number of users, with a per-user price beyond.

PK-01

Core Training Engineering Pack

\$99

per month • 100 users included • \$0.75 per extra user

Scope, evidence, diagnosis, the Training Engineering Report and the roadmap.

PK-04

Compliance & Certification Pack

\$99

per month • 300 users included • \$0.55 per extra user

Certificate tracking and readiness evidence for ISO 7101, JCI and ISO 15189.

PK-03

Assessment, Analytics & ROI Pack

\$129

per month • 500 users included • \$0.5 per extra user

Kirkpatrick evaluation, KPIs and return on investment.

PK-02

Learning Delivery & Experience Pack

\$149

per month • 250 users included • \$0.6 per extra user

Paths, modules, sessions and learner engagement.

Add-ons that fit hospital groups

Catalog prices per month, in USD.

Add-on	Price	What it adds
AD-08 • Multi-Site / Multi-Entity Deployment	\$55	Adapt and deploy training plans across multiple sites, entities, or business units, ensuring local relevance while maintaining strategic alignment.
AD-09 • External Training Plan Validation (Regulatory)	\$45	Manage external validation and regulatory compliance for training plans, including submission to regulatory bodies (e.g., OFPPT, GIAC-Agro).
AD-11 • Questionnaire Library & Multi-Channel Distribution	\$49	Provide pre-built questionnaire templates tailored to Focus (All training needs, Digital, AI, Custom) and sector, with multi-channel distribution (Face-to-Face, Email, Hybrid, Custom), RACSI allocation per section and structured response collection.
AD-01 • Immersive Learning (VR/AR/Simulation)	\$49	Create experiential, safe-to-fail learning environments using VR, AR, and simulation technologies.
AD-07 • Expert Knowledge Capture & Transfer	\$45	Preserve institutional knowledge before it walks out the door.

Bundle	Packs	Price
Starter Bundle	PK-01 + PK-02	\$223.2 (~10%)
Growth Bundle	PK-01 + PK-02 + PK-03	\$320.45 (~15%)
Enterprise Bundle	PK-01 to PK-05	\$476 (~20%)
Strategic Bundle	PK-01 to PK-06 + PK-08	\$677.25 (~25%)
Full Suite	PK-01 to PK-08	\$806.4 (~30%)

A first run in four steps

Sized by the complexity score: about 30, 60 or 90 days on an SME track; a full run for a hospital group.



01

Set up

Group, organization, functions, users and roles; connect the HR system.



02

Scope and evidence

Approve the Scope of Work, send questionnaires, pass gate G1.



03

Diagnose and plan

Five-axis diagnostic, gaps, prioritized themes, report, budget; gates G2 and G3.



04

Deliver and evaluate

Sessions, Kirkpatrick evaluation, certificates, return on experience; gates G4 to G6.

POWERACT supports each step: set-up, method coaching and the first gate reviews.

Start with one hospital, one run



Book a working session with POWERACT: your accreditation calendar, sites and priority roles.



See the demonstration: sign in to the healthcare tenant and follow a task from scope to certificate.



Run a pilot: one site or one clinic, Digital or AI focus, measured with the same KPIs.

POWERACT Consulting

www.poweract.ma

Figures in this presentation come from the demonstration instance. Organizations and people are fictional.